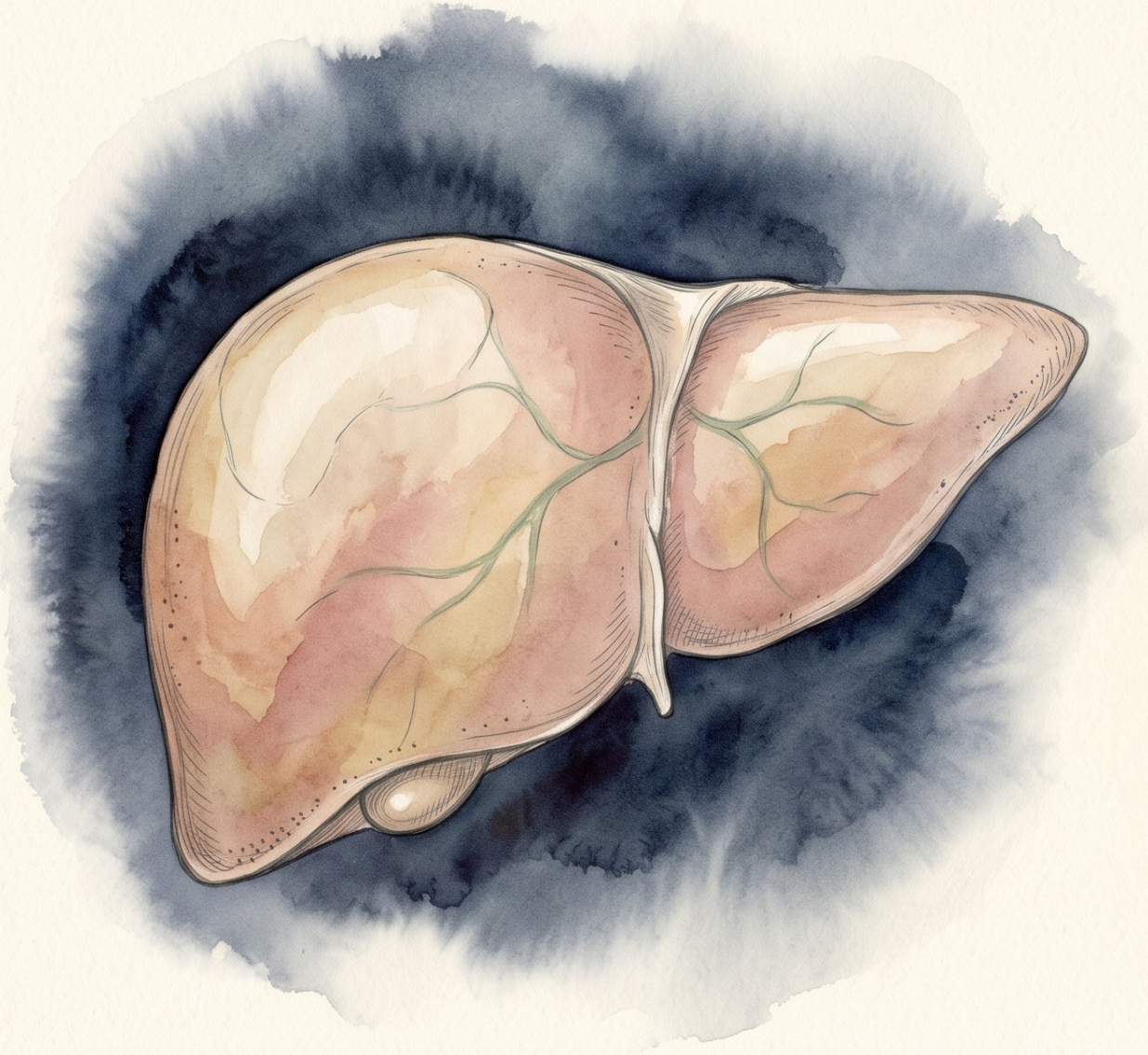


Acute Liver Failure and ACLF in the ICU

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Tufts University | School of Medicine

Slides (PDF)

Illustrations generated with Google Gemini

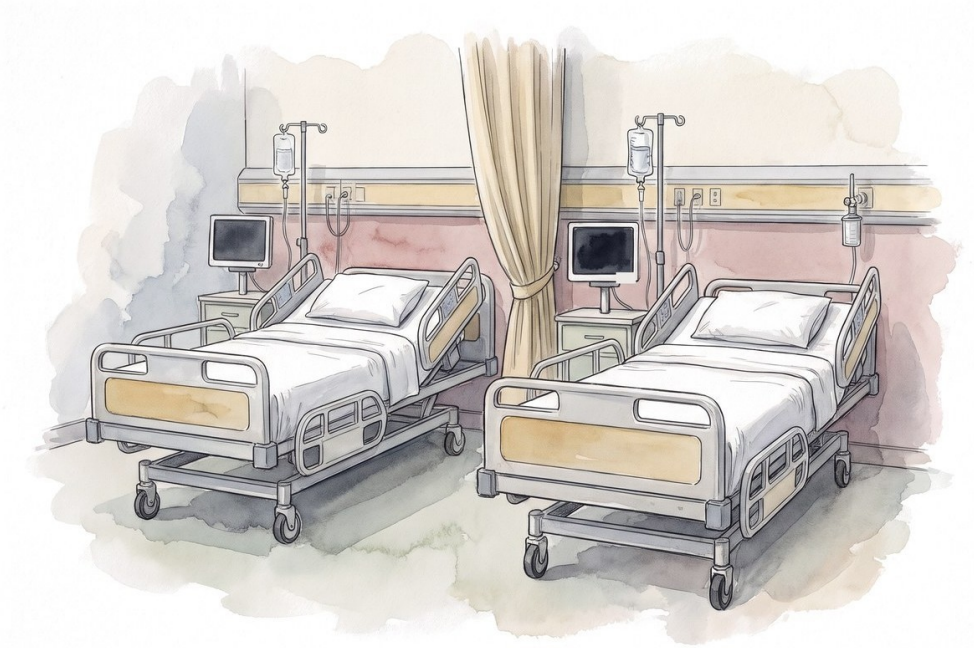
Objectives

By the end of this talk you should be able to:

- Tell acute liver failure apart from severe acute liver injury and from ACLF
- Plan the first hour of care in ALF
- Manage the brain and the ammonia in ALF
- Recognize and grade ACLF, and find the trigger
- Work through AKI in cirrhosis
- Know when to call transplant hepatology

Bed 4 and bed 7

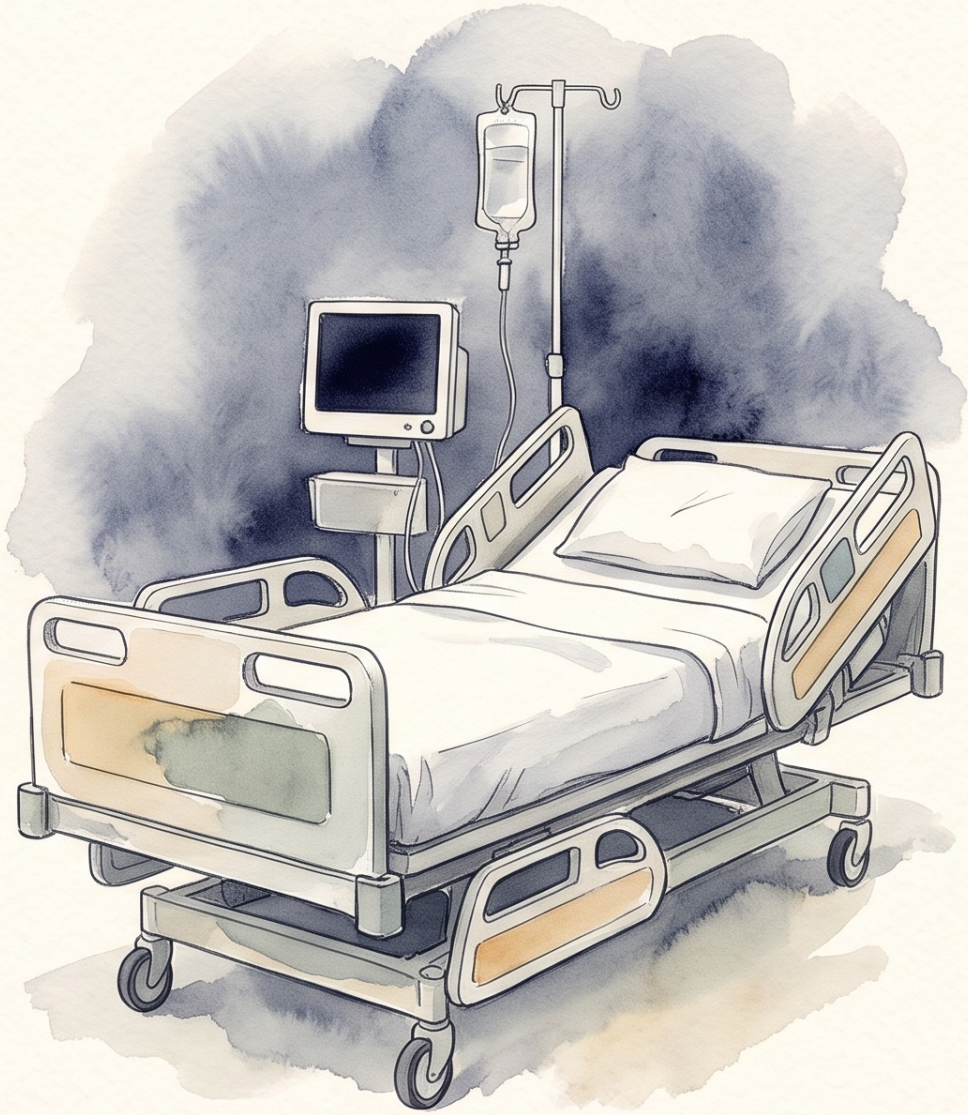
Bed 4	Bed 7
28-year-old woman	56-year-old man
No known liver disease	Alcohol-related cirrhosis with ascites
Nausea and vomiting for 4 days	Fever, confusion, low blood pressure



Is it the same disease?

Bed 4

28F, 4 days of nausea



28F, 4 days of nausea

- Nausea, vomiting and poor intake for 4 days
- No known liver disease, no alcohol use
- "Took something for a toothache"

Test	Result
AST	6,820 U/L
ALT	5,940 U/L
ALP	128 U/L
Bilirubin	2.9 mg/dL
Creatinine	1.0 mg/dL

Is this acute liver failure?

28F, 4 days of nausea

- Nausea, vomiting and poor intake for 4 days
- No known liver disease, no alcohol use
- "Took something for a toothache"

Test	Result
AST	6,820 U/L
ALT	5,940 U/L
ALP	128 U/L
Bilirubin	2.9 mg/dL
Creatinine	1.0 mg/dL

Is this acute liver failure?

We don't know yet. What's missing?

28F, 4 days of nausea

- Nausea, vomiting and poor intake for 4 days
- No known liver disease, no alcohol use
- "Took something for a toothache"

Test	Result
AST	6,820 U/L
ALT	5,940 U/L
ALP	128 U/L
Bilirubin	2.9 mg/dL
Creatinine	1.0 mg/dL

What's missing?

- **INR** (synthetic function, not enzymes)
- **Mental status** (any encephalopathy?)
- **Time course** (how long since the first symptom?)
- **Is there chronic liver disease underneath?**

28F, 4 days of nausea

- Disoriented to time, asterixis. Still there after the glucose is fixed
- Symptoms started 4 days ago
- Ultrasound: smooth liver, no ascites, patent vessels, normal spleen
- No history or imaging suggesting cirrhosis

Test	Result
AST	6,820 U/L
ALT	5,940 U/L
Bilirubin	2.9 mg/dL
INR	**3.2**
Glucose	**54 mg/dL**

28F, 4 days of nausea

- Disoriented to time, asterixis. Still there after the glucose is fixed
- Symptoms started 4 days ago
- Ultrasound: smooth liver, no ascites, patent vessels, normal spleen
- No history or imaging suggesting cirrhosis

Test	Result
AST	6,820 U/L
ALT	5,940 U/L
Bilirubin	2.9 mg/dL
INR	**3.2**
Glucose	**54 mg/dL**

This is acute liver failure

Acute liver failure

- **INR ≥ 1.5** and **any degree of encephalopathy**
- Illness of less than 26 weeks
- No pre-existing cirrhosis
  - AASLD and ACG: chronic liver disease without cirrhosis still allows ALF. EASL: no prior liver disease at all
  - Wilson disease, autoimmune hepatitis and vertically acquired HBV can still count if recognized within 26 weeks

Jaundice to encephalopathy	Hyperacute (< 1 week)	Acute (1 to 4 weeks)	Subacute (4 to 12 weeks)
Typical causes	Acetaminophen, HAV, HEV, ischemia	HBV	DILI, AIH, unknown cause
Cerebral edema	**Common**	Less common	Uncommon
Spontaneous recovery	More likely	Intermediate	**Less likely**

EASL ALF guideline 2017 [PMID: 28417882]; ACG ALF 2023 [PMID: 37377263]; Management of ALF: update 2022 [PMID: 36001996]

Same patient, but wide awake

Same labs and INR 2.4, but fully oriented with no asterixis

What do you call this, and what do you do now?



Same patient, but wide awake

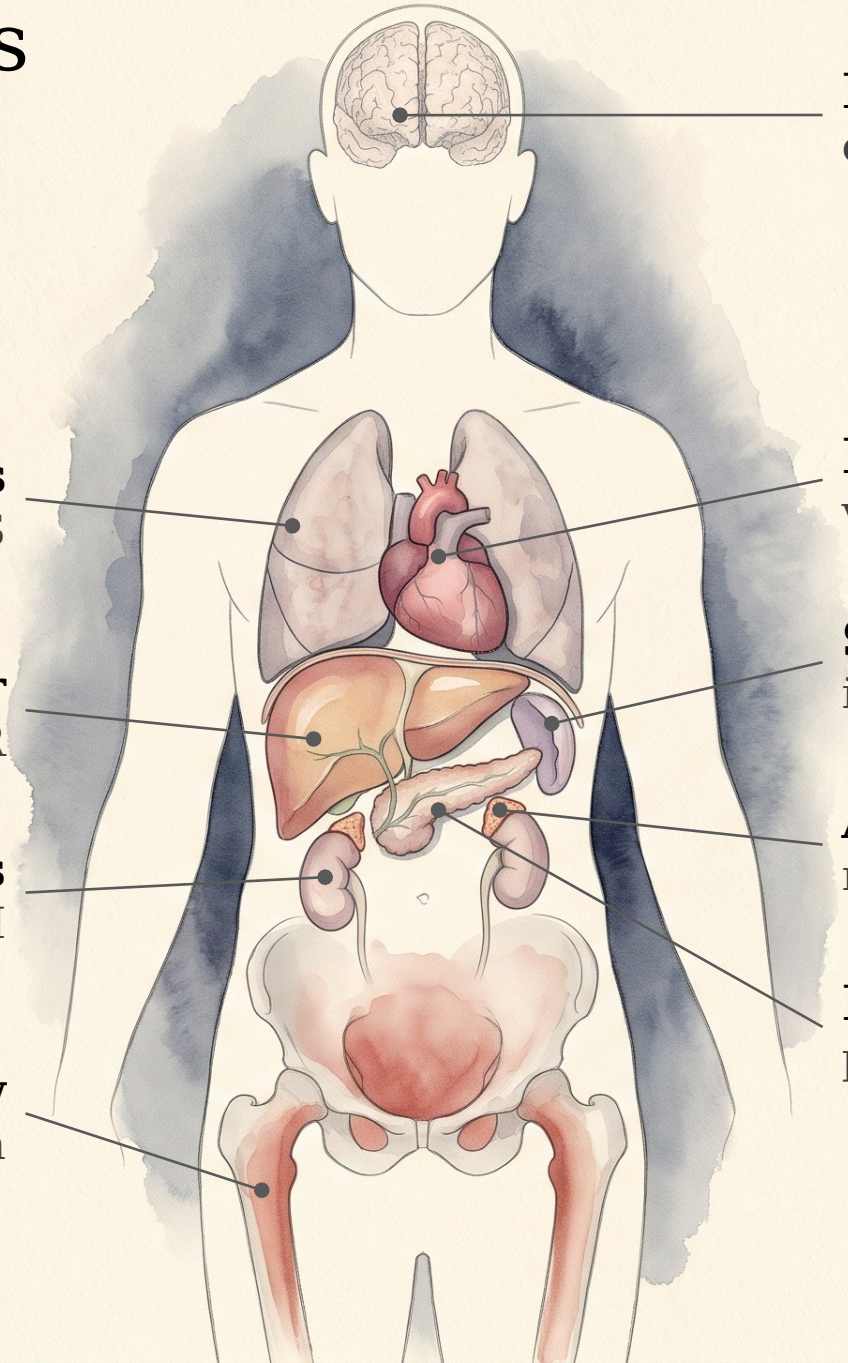
Same labs and INR 2.4, but fully oriented with no asterixis

- **Severe acute liver injury**, not ALF yet
- No encephalopathy, so it doesn't meet the definition
- But it can become ALF within hours
  - Check glucose often and examine the patient every few hours
- And the opposite trap:
  - Hypoglycemia, sedatives, sepsis, alcohol withdrawal and seizures can all look like encephalopathy

Which other organs fail in ALF?



Which other organs fail in ALF?



Brain
encephalopathy, cerebral edema

Lungs
ARDS

Heart and vessels
vasodilatory shock

Liver
hypoglycemia, lactate, INR

Spleen and immune system
infection

Kidneys
AKI

Adrenals
relative insufficiency

Pancreas
pancreatitis

Bone marrow
suppression

Figure content after Bernal W, Wendon J. N Engl J Med 2013;369:2525-34

28F, 4 days of nausea

What's your next step?

The first hour

Support	Workup
ICU bed	Acetaminophen level
Glucose every 1 to 2 hours	Viral, autoimmune, Wilson, toxicology
Volume, perfusion, lactate	Doppler ultrasound
Neuro checks	Pregnancy test

Send the workup while you resuscitate

Which cause changes treatment right away?

If you suspect	Clues	Send	Don't wait to
Acetaminophen	AST/ALT in the thousands, OTC use	Level, combination products	
Herpes simplex	Fever, very high AST/ALT, little jaundice	HSV PCR (blood)	
Hepatitis B	Endemic area, chemo or immunosuppression	HBsAg, anti-HBc IgM, DNA	
Autoimmune hepatitis	Woman, other autoimmune disease	ANA, ASMA, IgG	
Wilson disease	Under 40, Coombs-negative hemolysis	Ceruloplasmin, urine copper	
AFLP, HELLP	Third trimester, high BP, low platelets	hCG	
Budd-Chiari	Pain, ascites, big liver	Doppler or CT	
Ischemic hepatitis	Shock or arrest, very high LDH	Echo	
Amanita	Wild mushrooms, diarrhea 6-24 h later	Poison control	

Update 2022 [PMID: 36001996]; practical update 2024 [PMID: 39170946]; EASL AIH 2025 [PMID: 40348684]

Which cause changes treatment right away?

If you suspect	Clues	Send	Don't wait to
Acetaminophen	AST/ALT in the thousands, OTC use	Level, combination products	Start NAC
Herpes simplex	Fever, very high AST/ALT, little jaundice	HSV PCR (blood)	Start IV acyclovir
Hepatitis B	Endemic area, chemo or immunosuppression	HBsAg, anti-HBc IgM, DNA	Entecavir or tenofovir
Autoimmune hepatitis	Woman, other autoimmune disease	ANA, ASMA, IgG	Steroids with hepatology
Wilson disease	Under 40, Coombs-negative hemolysis	Ceruloplasmin, urine copper	Urgent transplant
AFLP, HELLP	Third trimester, high BP, low platelets	hCG	Obstetrics: delivery
Budd-Chiari	Pain, ascites, big liver	Doppler or CT	Anticoagulate; TIPS
Ischemic hepatitis	Shock or arrest, very high LDH	Echo	Fix the circulation
Amanita	Wild mushrooms, diarrhea 6-24 h later	Poison control	NAC, silibinin

Update 2022 [PMID: 36001996]; practical update 2024 [PMID: 39170946]; EASL AIH 2025 [PMID: 40348684]

28F, 4 days of nausea

- Roommate: "She was taking a lot of the Tylenol with codeine, plus cold medicine, for 3 days"
- Acetaminophen level: **12 µg/mL**, time of last dose unknown

Can you use the nomogram?



28F, 4 days of nausea

- Roommate: "She was taking a lot of the Tylenol with codeine, plus cold medicine, for 3 days"
- Acetaminophen level: **12 µg/mL**, time of last dose unknown



Can you use the nomogram?

- **No.** Rumack-Matthew is for a single acute ingestion with a known time
- Repeated supratherapeutic use and late presentation: the level can be low or undetectable
- **Start NAC now**

NAC for every kind of acute liver failure?

Published in final edited form as:
Gastroenterology. 2009 September ; 137(3): 856–864.e1. doi:10.1053/j.gastro.2009.06.006.

WM Lee[1], **LS Hynan**[1], **L Rossaro**[2], **RJ Fontana**[3], **RT Stravitz**[4], **AM Larson**[5], **TL Davern II**[6], **NG Murray**[7], **T McCashland**[8], **JS Reisch**[1], **PR Robuck**[9], and **the Acute Liver Failure Study Group**

What do you think they found?

NAC for every kind of acute liver failure?

Published in final edited form as:
Gastroenterology. 2009 September ; 137(3): 856–864.e1. doi:10.1053/j.gastro.2009.06.006.

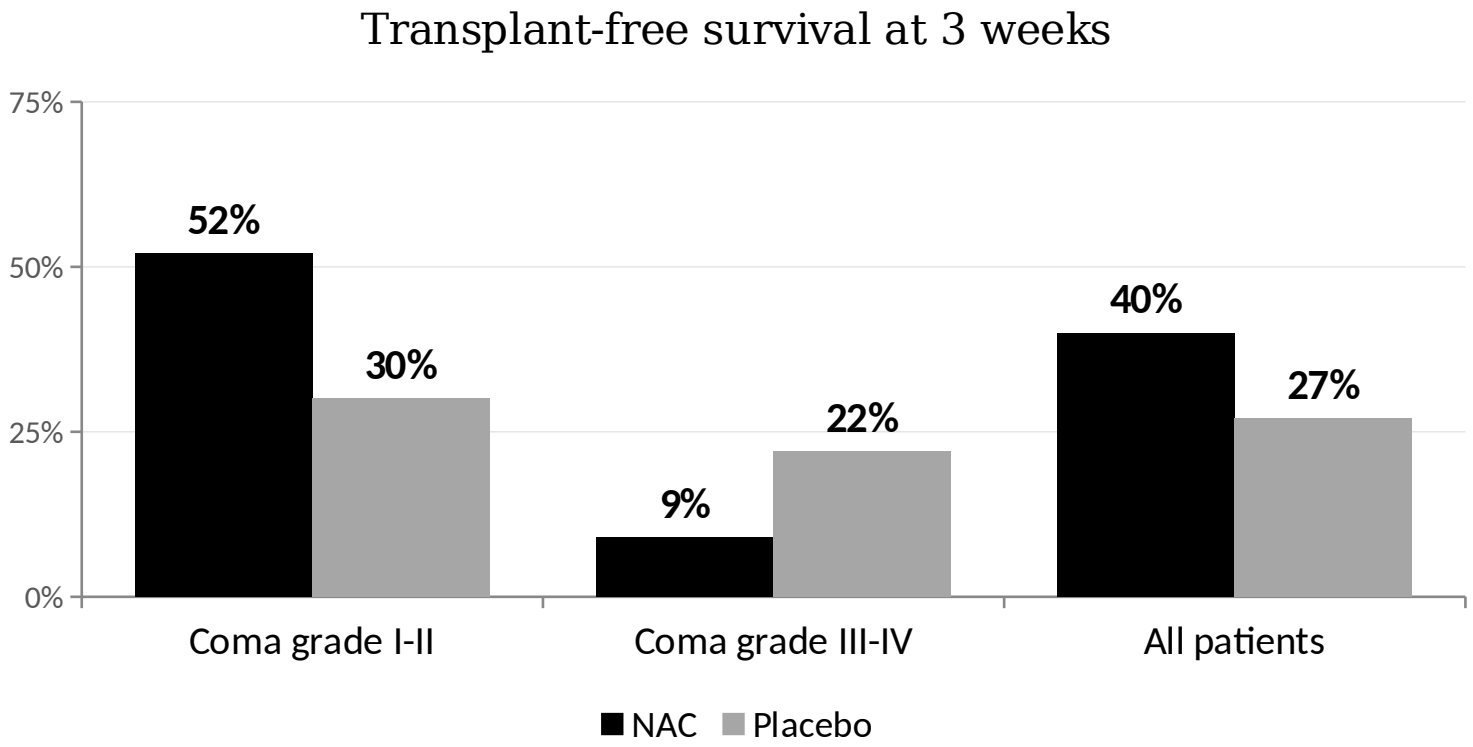
INTRAVENOUS *N*-ACETYLCYSTEINE IMPROVES TRANSPLANT-FREE SURVIVAL IN EARLY STAGE NON-ACETAMINOPHEN ACUTE LIVER FAILURE

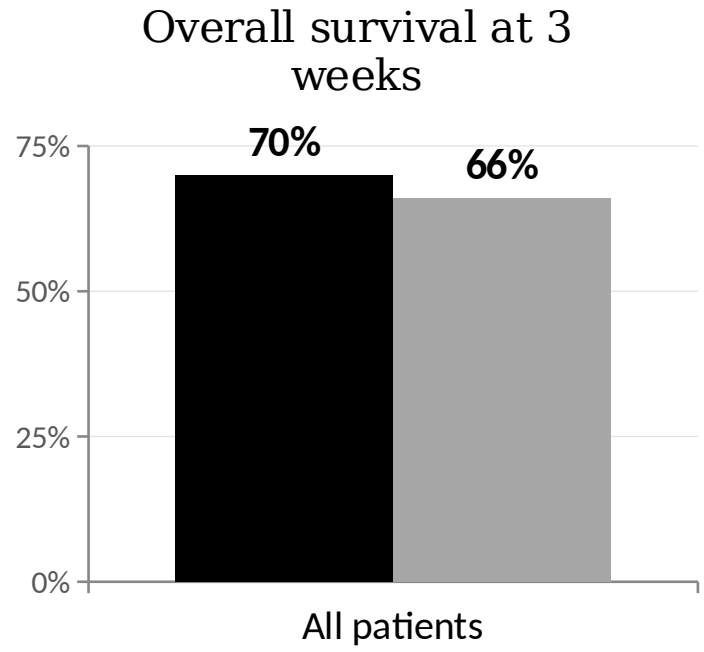
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Conclusions—Intravenous NAC improves transplant-free survival in patients with early stage non-acetaminophen related acute liver failure. Patients with advanced coma grades do not benefit from NAC and typically require emergency liver transplantation. (ClinicalTrials.gov number NCT00004467)

NAC for every kind of acute liver failure?

Adult non-acetaminophen ALF, randomized, NAC vs placebo, 173 patients





Reasonable in early non-acetaminophen ALF. Don't extrapolate to advanced coma, children or ACLF

Lee WM et al. Gastroenterology 2009 [PMID: 19524577]

28F, 4 days of nausea

	Admission	Hour 12
Mental status	Disoriented, asterixis	**Responds only to voice (grade III)**
Ammonia (arterial)	Not sent	**182 µmol/L**
Urine output	OK	15 mL/h for 4 hours
Lactate (after 3 L)	2.1 mmol/L	4.6 mmol/L
Creatinine	1.0 mg/dL	1.3 mg/dL
Sodium	136 mmol/L	133 mmol/L
Glucose	54 mg/dL	Needs D10

What worries you most?

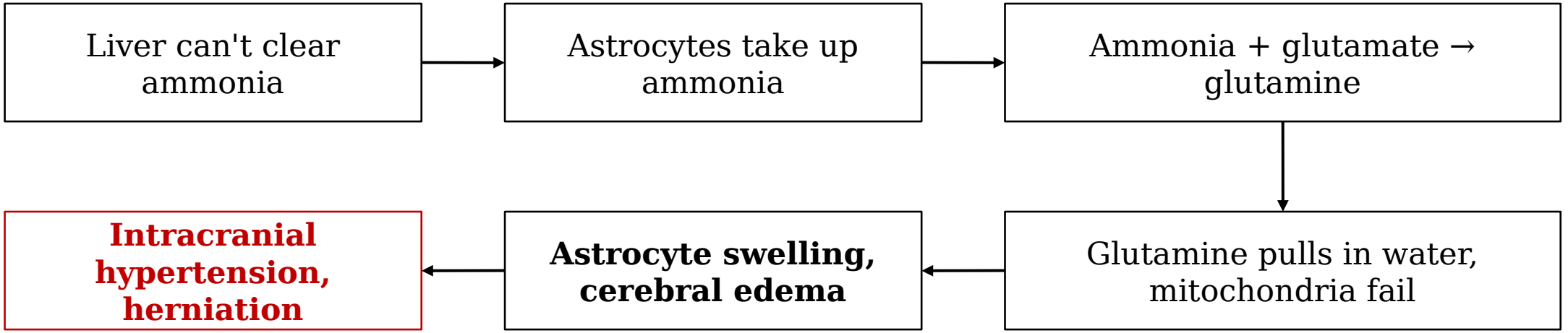
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Glucose	54 mg/dL	Needs D10

Her brain

When do you intubate? Do you wait for the creatinine before dialysis?

Why does the brain swell?



- **Makes it worse:** inflammation and infection, loss of autoregulation, fever, low sodium
- **Highest risk:** hyperacute course, young, grade III-IV encephalopathy, ammonia persistently above 150 to 200 µmol/L, AKI, vasopressors

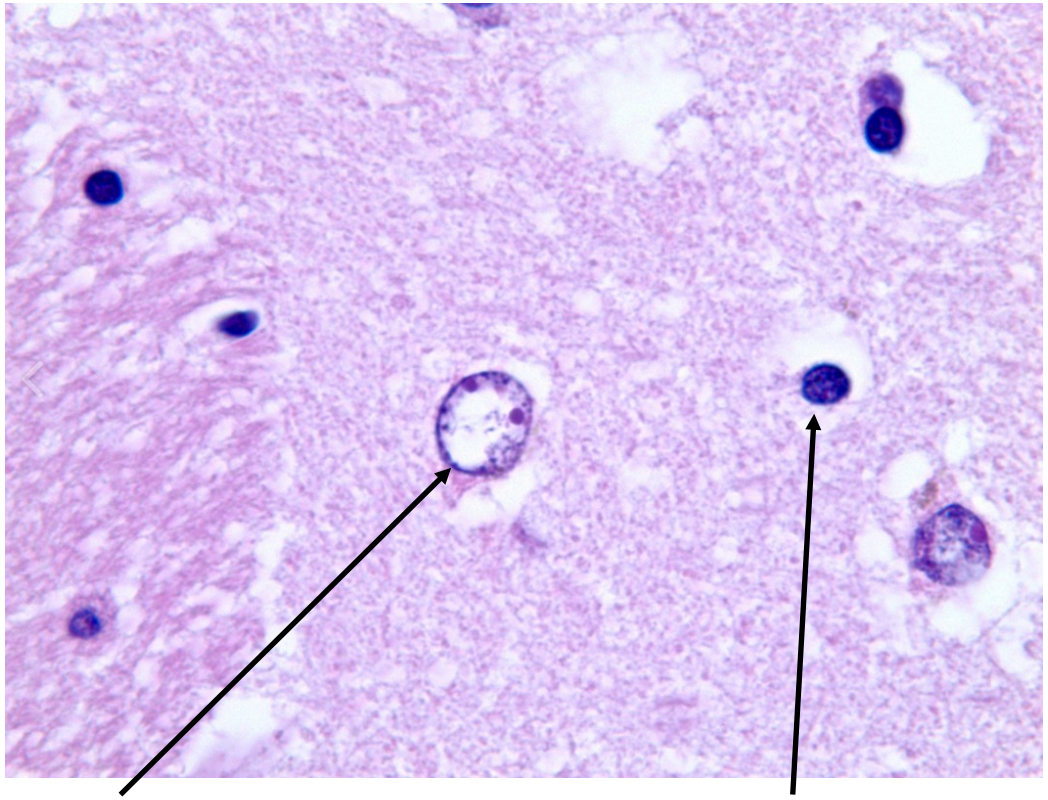
Update 2022 [PMID: 36001996]; practical update 2024 [PMID: 39170946]

Ammonia: ALF vs cirrhosis



Why do patients with cirrhosis rarely get cerebral edema, even with a high ammonia?

Ammonia: ALF vs cirrhosis



Alzheimer type II astrocyte: big, pale nucleus

Normal glial nucleus: small and dark

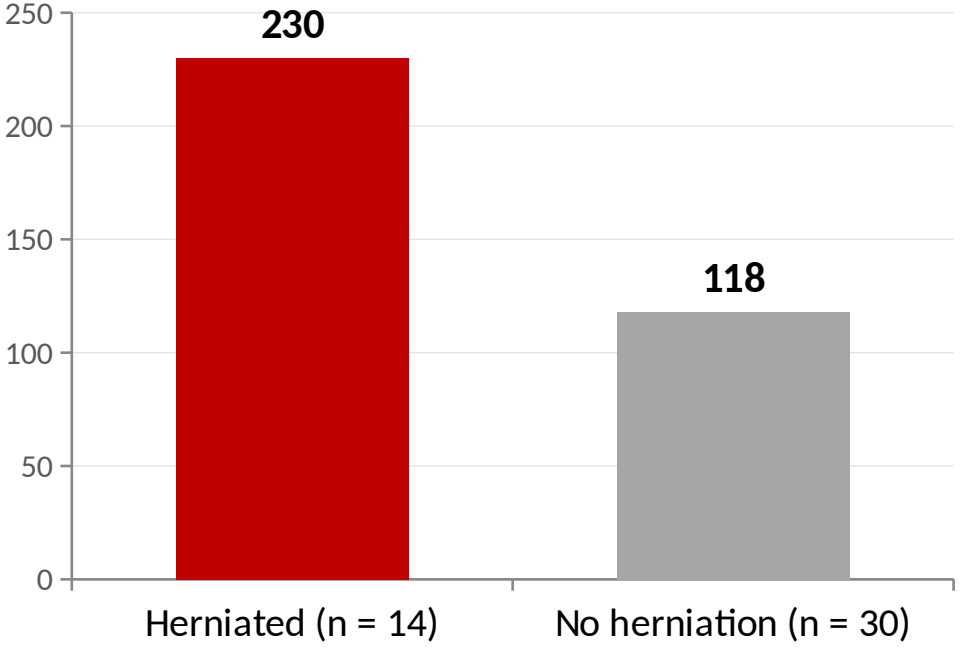
- Astrocytes clear ammonia by turning it and glutamate into glutamine
- Glutamine pulls water into the cell
- **Cirrhosis:** over months the astrocyte pushes out other solutes (myo-inositol, taurine) to make room. It swells only a little. This is the cell you see under the microscope
- **ALF:** the ammonia comes in days. No time to adapt, so the astrocytes swell and the brain swells with them

Cerebral edema is common in hyperacute ALF and rare in cirrhosis

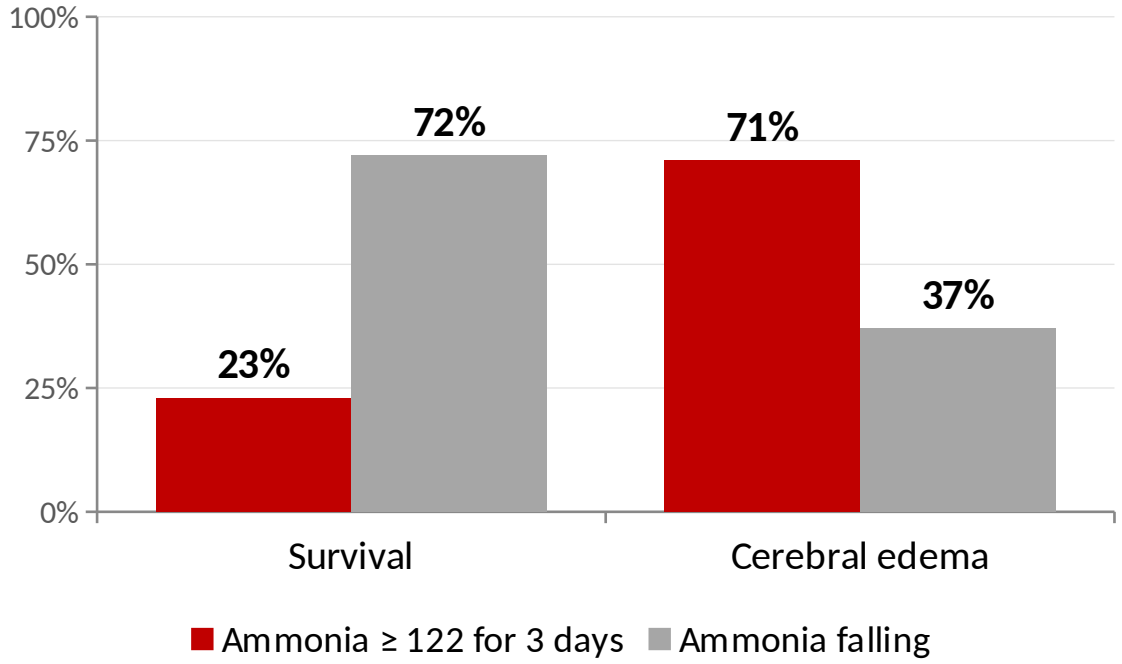


Ammonia: how high is too high?

Clemmesen 1999: ammonia, µmol/L



Kumar 2012: 295 patients



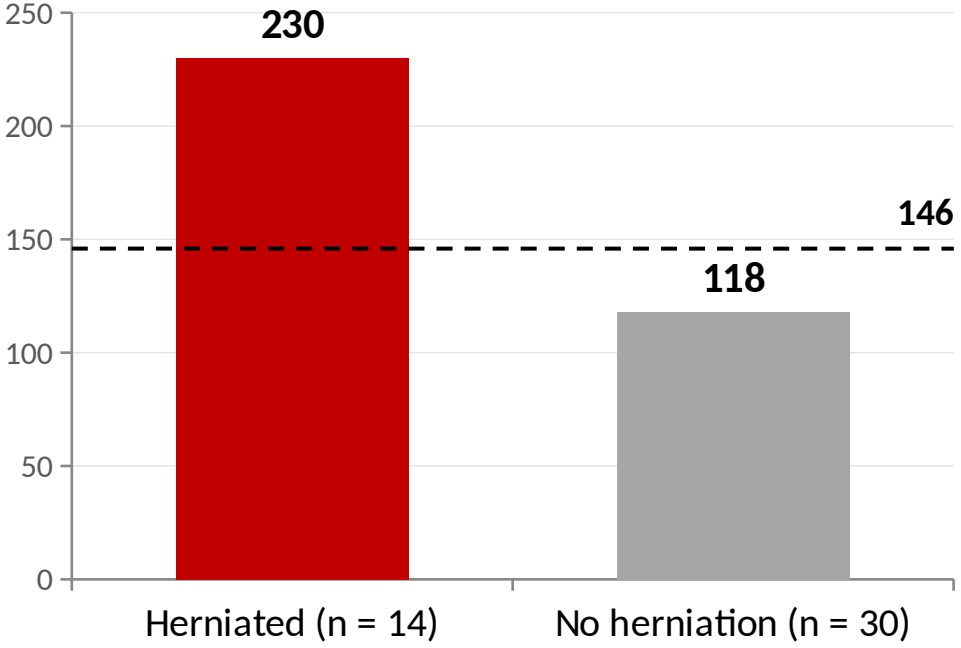
So what number worries you?

Clemmesen JO et al. Hepatology 1999 [PMID: 10051463]; Kumar R et al. Clin Gastroenterol Hepatol 2012 [PMID: 22521861]; more studies in the appendix

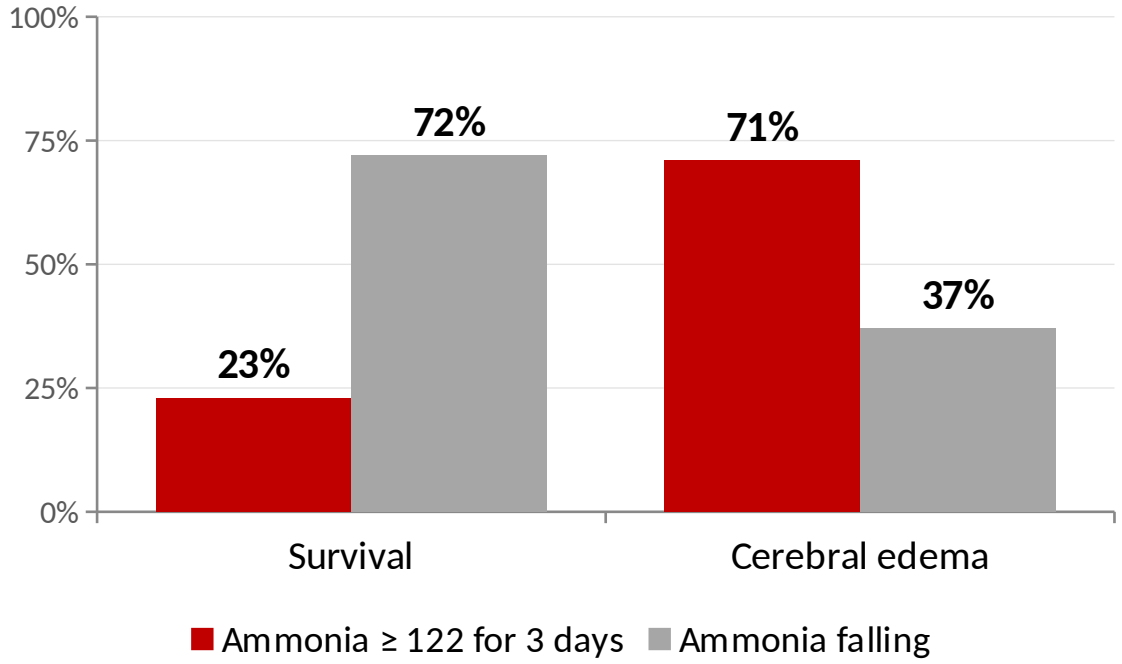


Ammonia: how high is too high?

Clemmesen 1999: ammonia, µmol/L



Kumar 2012: 295 patients



Above 150 to 200 and not falling: worry about the brain

No single level rules ICH in or out. Read it with the HE grade and the trend

Clemmesen JO et al. Hepatology 1999 [PMID: 10051463]; Kumar R et al. Clin Gastroenterol Hepatol 2012 [PMID: 22521861]; more studies in the appendix

Lactulose for her?

Hour 12: ammonia 182 µmol/L, grade III. In a cirrhotic we'd already be giving it

Yes or no?



Lactulose for her?

Hour 12: ammonia 182 µmol/L, grade III. In a cirrhotic we'd already be giving it

Why people give it	Why not
Ammonia drives brain swelling	**No proven benefit in ALF (ALFSG)**
It's what we do in cirrhosis	Distended bowel gets in the way at transplant
Cheap; AASLD 2011 allowed it early	Diarrhea: free water loss, sodium swings
	Liquid by mouth or tube: aspiration risk

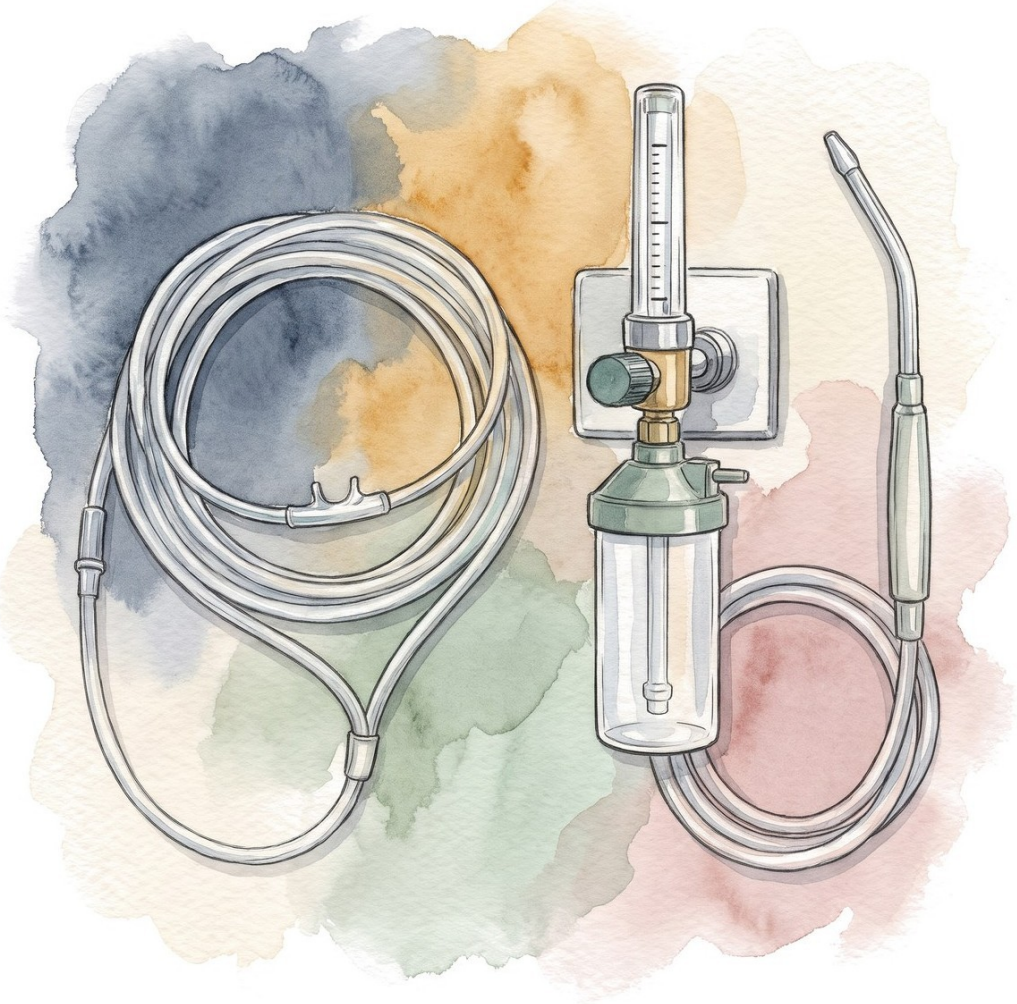
Not for her

AASLD position paper 2011, rec. 26; Alba L, Hay JE, Lee WM et al. J Hepatol 2002;36(Suppl 1):33 (abstract); ACG ALF 2023 [PMID: 37377263]

When do you intubate?

- Responds to voice, drifts off, intermittently agitated
- Handling secretions for now
- SpO2 96% on 2 L

Intubate now, or wait until she can't protect her airway?



When do you intubate?

Grade III-IV encephalopathy in ALF: intubate

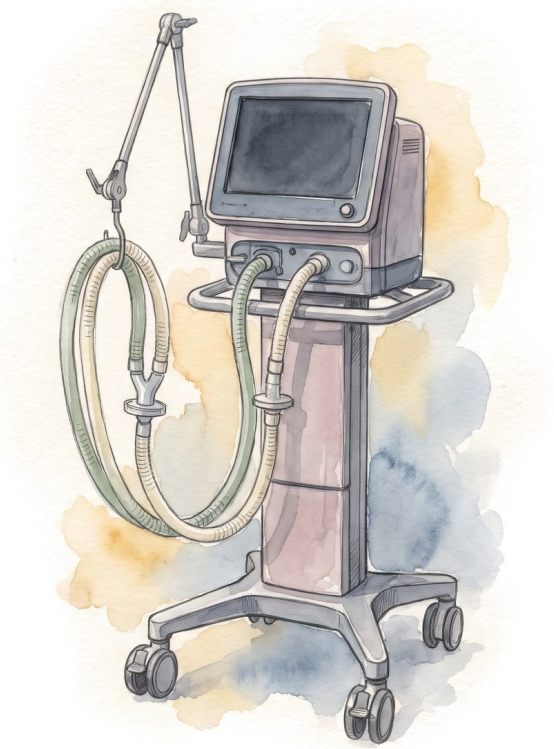
Before	During	After
Preoxygenate Norepinephrine ready Check glucose Head of bed 30°, neck neutral	Avoid hypotension (CPP) No coughing or bucking (ICP) Most experienced operator	Propofol: short-acting, may lower ICP Avoid long-acting benzodiazepines Check pupils: sedation hides the exam

Practical update 2024 [PMID: 39170946]; ACG ALF 2023 [PMID: 37377263]; SCCM 2020 [PMID: 32058387]

Can we use permissive hypercapnia?

After intubation: bilateral infiltrates, P/F 150 on PEEP 10

Lung protection or brain protection?



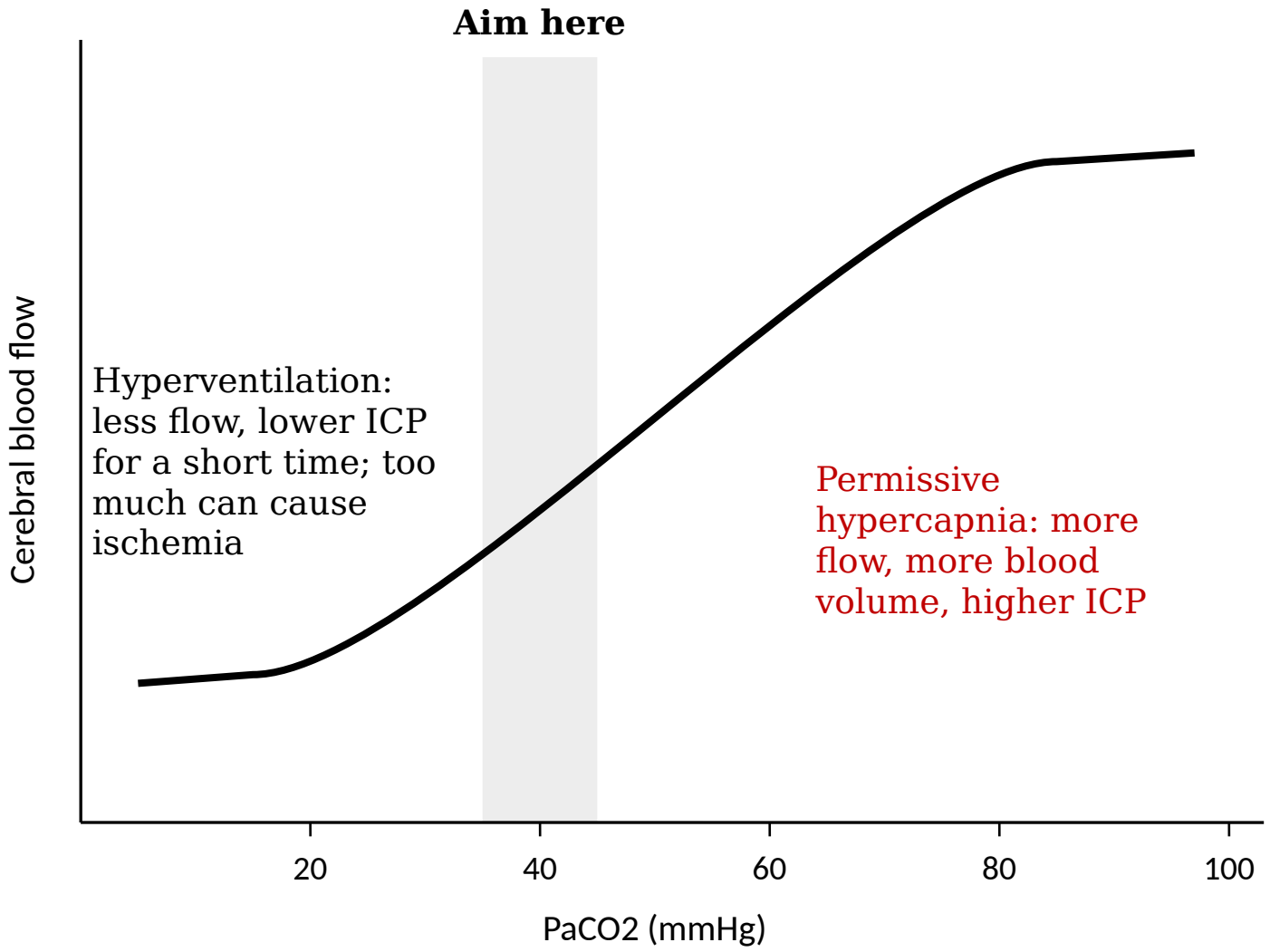
Can we use permissive hypercapnia?

After intubation: bilateral infiltrates, P/F 150 on PEEP 10

	For the lungs (ARDS)	For the brain (ICP)	What we do
Tidal volume	6 mL/kg PBW	Watch the CO2	6 mL/kg, raise the rate
CO2	Tolerate a high CO2	High CO2 raises ICP	**Keep the CO2 normal**
Hyperventilation	No	Lowers ICP briefly	Short rescue for herniation only
PEEP	Recruit	High PEEP: less venous return	Set for O2 and BP; PEEP is fine

SCCM 2020 [PMID: 32058387]; practical update 2024 [PMID: 39170946]

CO2 and cerebral blood flow

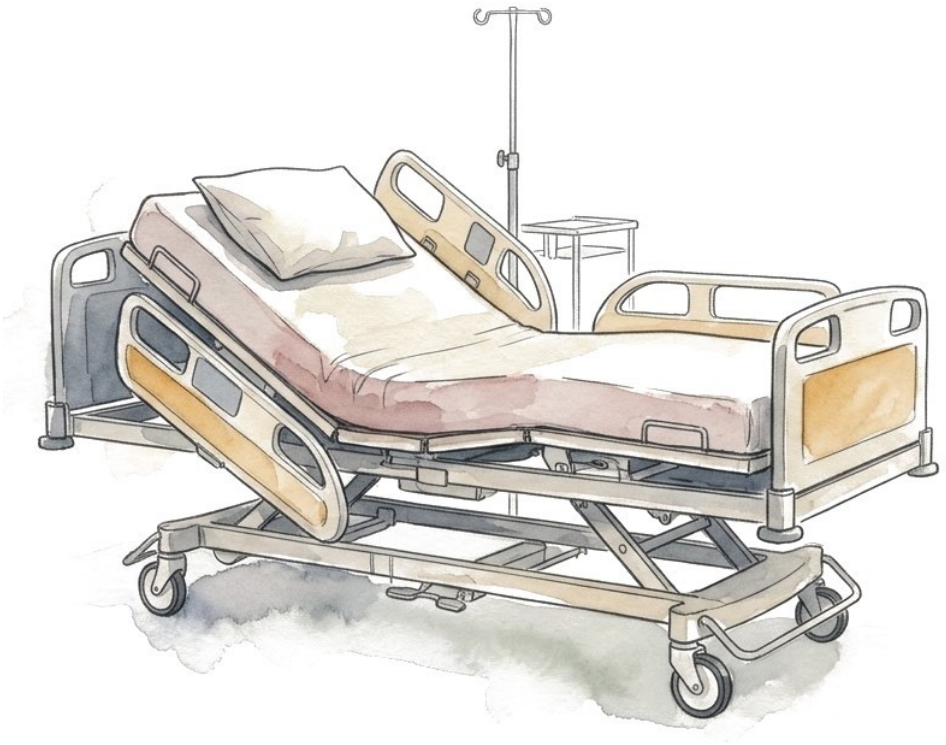


- Between about 20 and 80 mmHg, cerebral blood flow rises with PaCO2
- In a swollen brain, more blood volume means a higher ICP
- Keep PaCO2 normal. Hyperventilate only as a short bridge for herniation

Schematic, not to scale. Standard cerebral physiology

Protecting the brain: every patient with grade III-IV

- Head of bed 30°, neck neutral
- Treat fever, avoid hypoglycemia
- Sodium **145 to 155 mmol/L** with hypertonic saline (use your center's protocol)
- Avoid fluid overload
- Treat seizures, EEG if in doubt. No routine prophylaxis
- No cooling: 34 °C did not prevent intracranial hypertension in an RCT



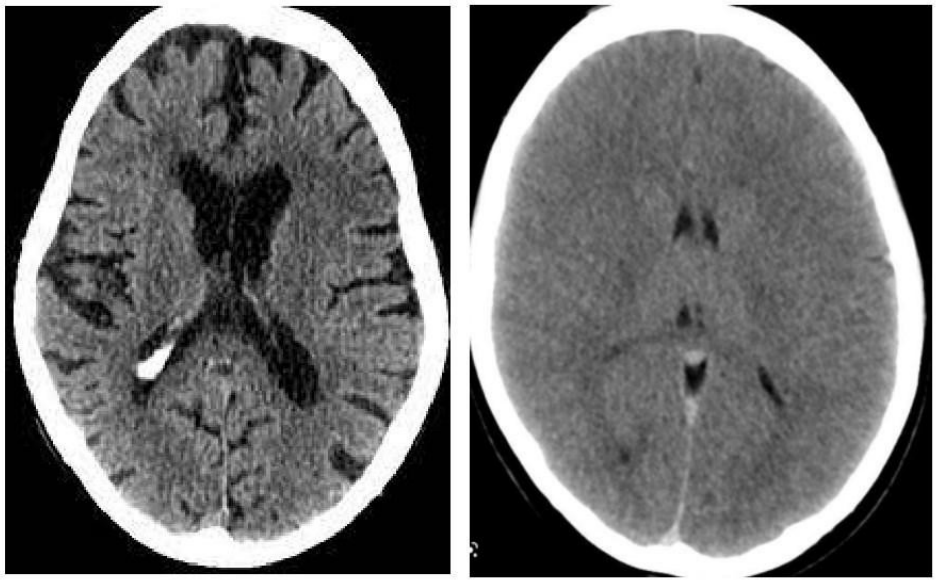
SCCM 2023 [PMID: 37052436]; update 2022 [PMID: 36001996]; Bernal W et al. J Hepatol 2016;65:273-9

Rising ICP: hypertension, bradycardia, a new pupil

- Hyperosmolar bolus (hypertonic saline or mannitol)
- Brief hyperventilation as a bridge
- Call the transplant team now

Things to know

- Hypertension and bradycardia come late. Don't wait for them
- A normal head CT does not exclude raised ICP
- ICP monitors are not routine; SCCM suggests against (very low certainty)



Normal | Effacement of sulci

SCCM 2023 [PMID: 37052436]; practical update 2024 [PMID: 39170946]

The creatinine is only 1.3

Ammonia 182 → 214 µmol/L, grade III, urine output 15 mL/h

Do you wait for a conventional dialysis indication?

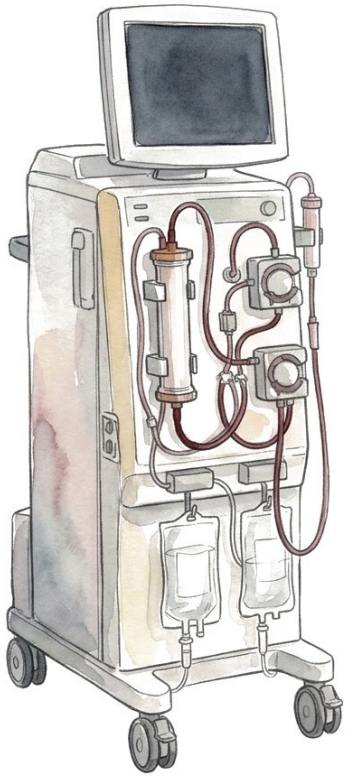


The creatinine is only 1.3

Ammonia 182 → 214 µmol/L, grade III, urine output 15 mL/h

No. In ALF, start CRRT for the ammonia

- ACG 2023: suggests early CRRT for hyperammonemia with grade II or higher encephalopathy, even without a renal indication (conditional, very low certainty)
- Continuous, not intermittent: fewer swings in blood pressure and brain water
- Call nephrology now



ACG ALF guideline 2023 [PMID: 37377263]; update 2022 [PMID: 36001996]

Should we correct this INR?

Hour 12: INR 4.1, platelets 88,000. She needs a dialysis catheter

2 units of FFP first?



Should we correct this INR?

Hour 12: INR 4.1, platelets 88,000. She needs a dialysis catheter

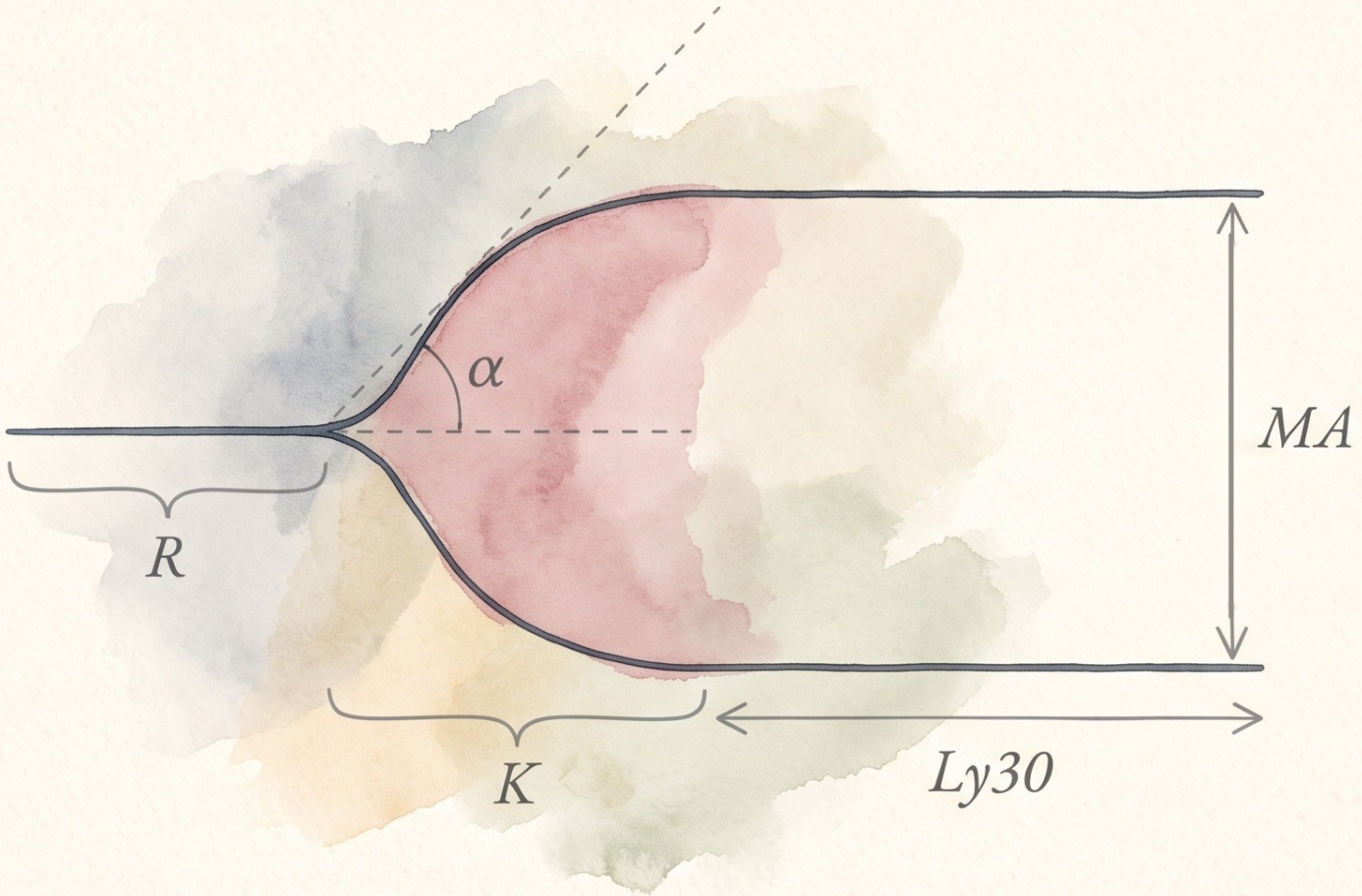
- **No plasma just to fix the number**
- The INR measures only part of clotting, and in ALF it doesn't predict who bleeds
- Plasma adds volume (lungs, brain) and erases the INR trend that prognosis and listing rely on

What to do instead

- Ultrasound-guided line, experienced operator, compressible site
- Vitamin K once; check fibrinogen and platelets; viscoelastic testing if available
- Treat bleeding. Plan high-risk procedures (like an ICP monitor) separately

Update 2022 [PMID: 36001996]; SCCM 2020 [PMID: 32058387]

Is her blood thin?

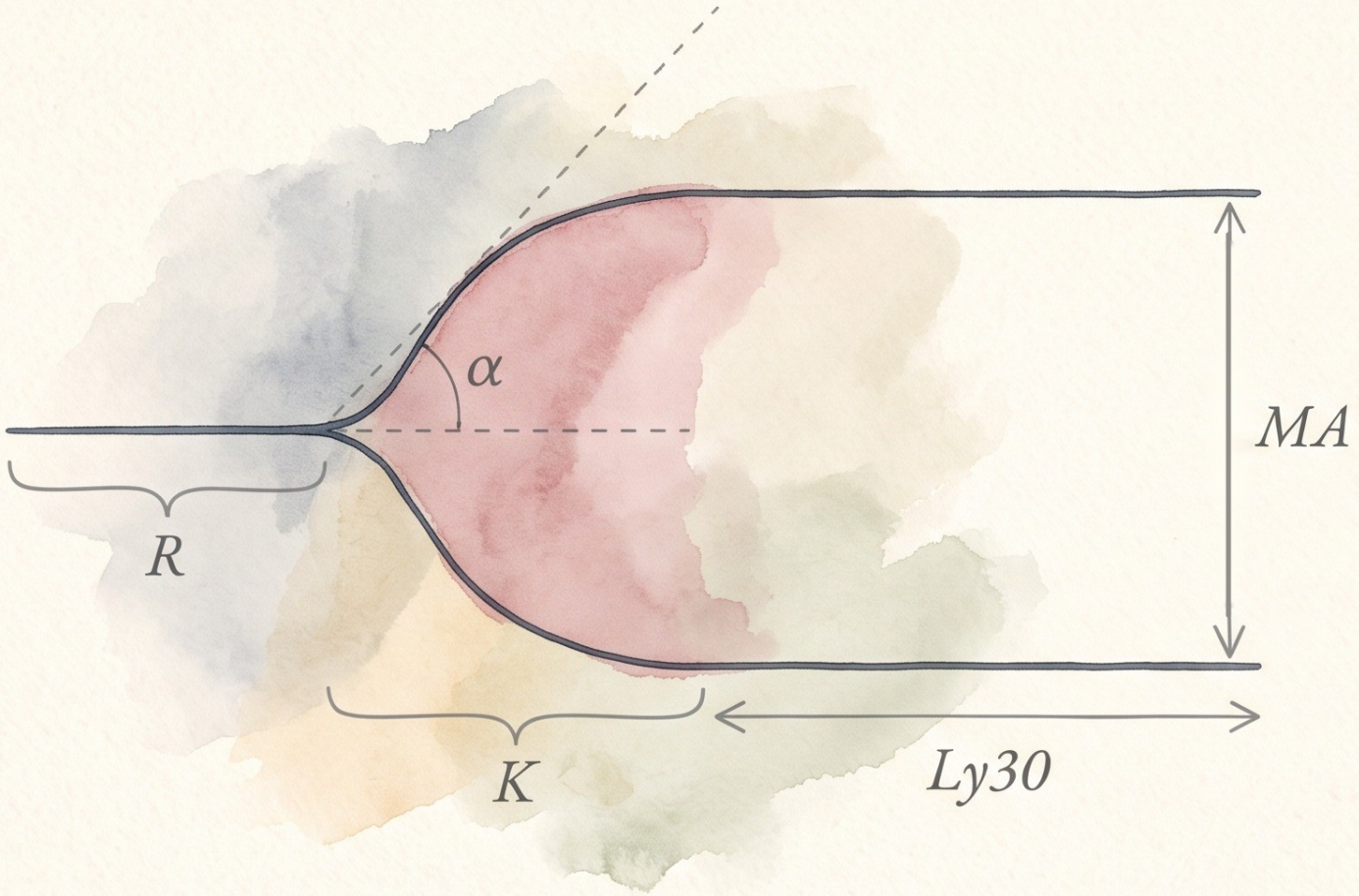


What a TEG measures in whole blood

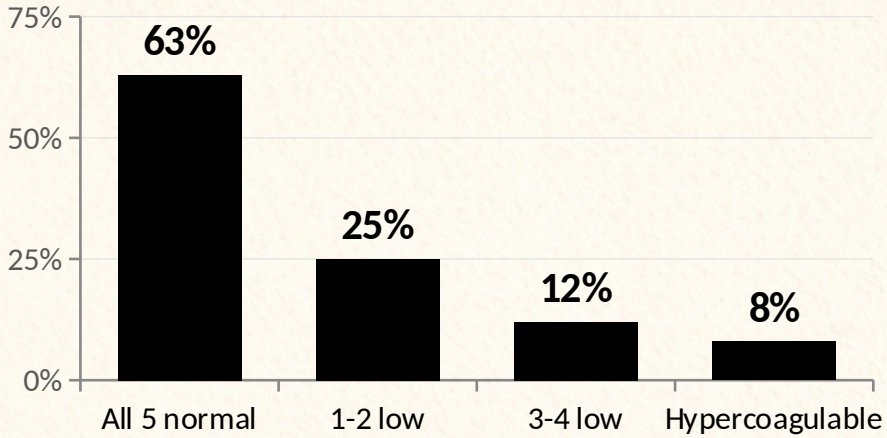
- **R:** time until the clot starts (clotting factors)
- **K and α-angle:** how fast the clot builds (fibrinogen)
- **MA:** how strong the clot gets (platelets, fibrinogen)
- **Ly30:** how much of the clot breaks down in 30 minutes

Her INR is 4.1. What will her TEG show?

Is her blood thin?



51 patients, mean INR 3.4



- Most had a completely normal TEG, even with INRs up to 9.6
- A weak clot (low MA) only when platelets were below about 126,000
- Encephalopathy: stronger clots, not weaker

A high INR doesn't mean she will bleed

Stravitz RT et al. J Hepatol 2012;56:129-36 [PMID: 21703173]

Rebalanced hemostasis

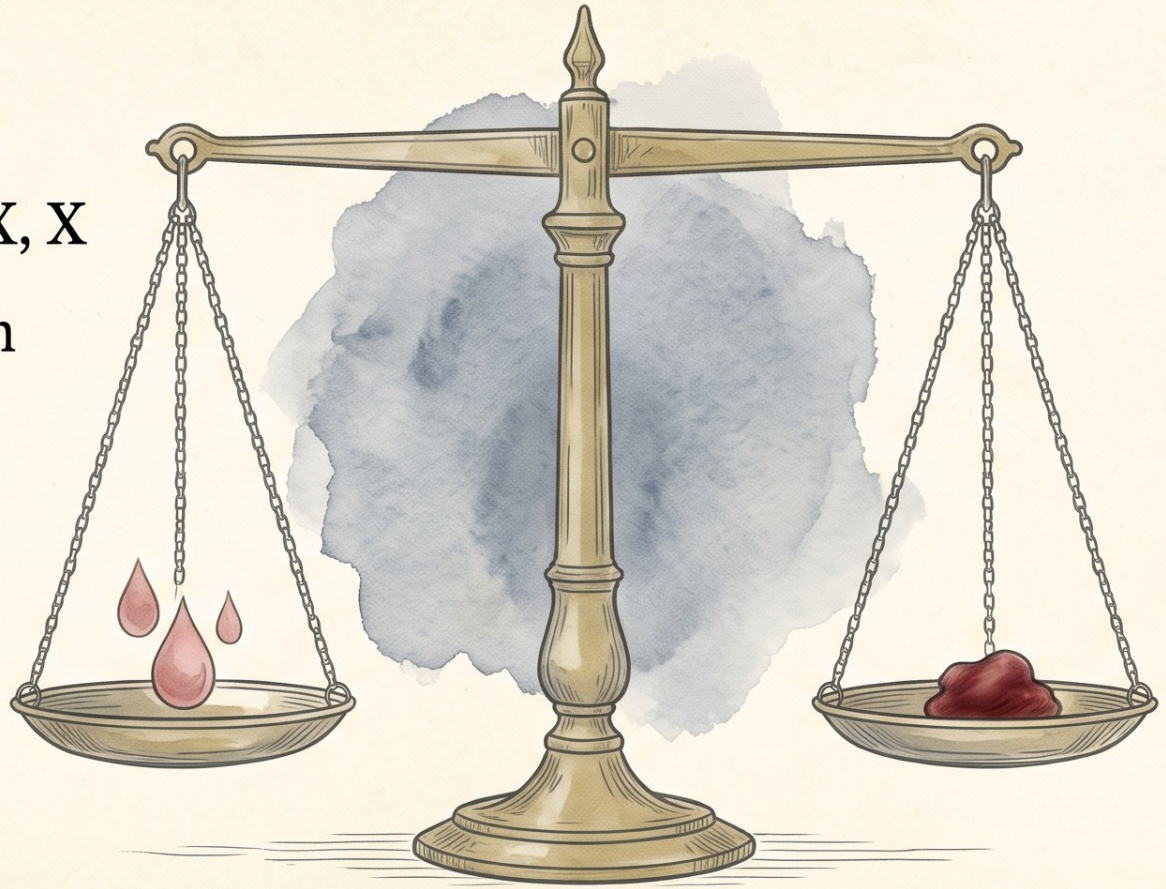
Favors bleeding

Low factors II, V, VII, IX, X

Low fibrinogen when severe

Low platelets

The INR sees part of this side



Favors clotting

Low protein C and protein S

Low antithrombin

High factor VIII and von Willebrand factor

The INR doesn't see this side

Balanced in most patients, but easily tipped either way: both bleeding and thrombosis happen

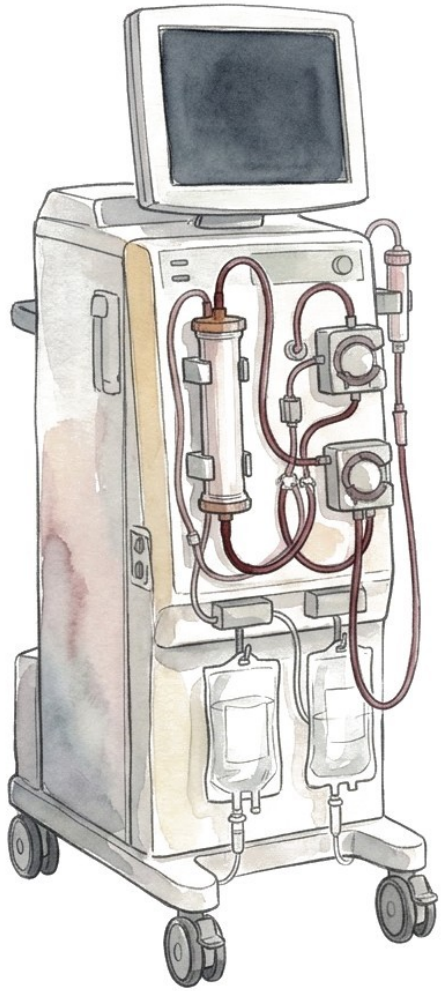
Concept from Lisman T, Porte RJ. Blood 2010

Calling nephrology

What do you ask them for?

Calling nephrology

- Indication: ammonia 214 and grade III encephalopathy. The creatinine doesn't matter here
- Start now and keep it running. A clotted filter means the ammonia climbs back
- One sodium plan: the CRRT fluid and the hypertonic saline together
- Watch phosphate and potassium, and calcium if they use citrate



ACG ALF 2023 [PMID: 37377263]; update 2022 [PMID: 36001996]

I know what you're thinking...

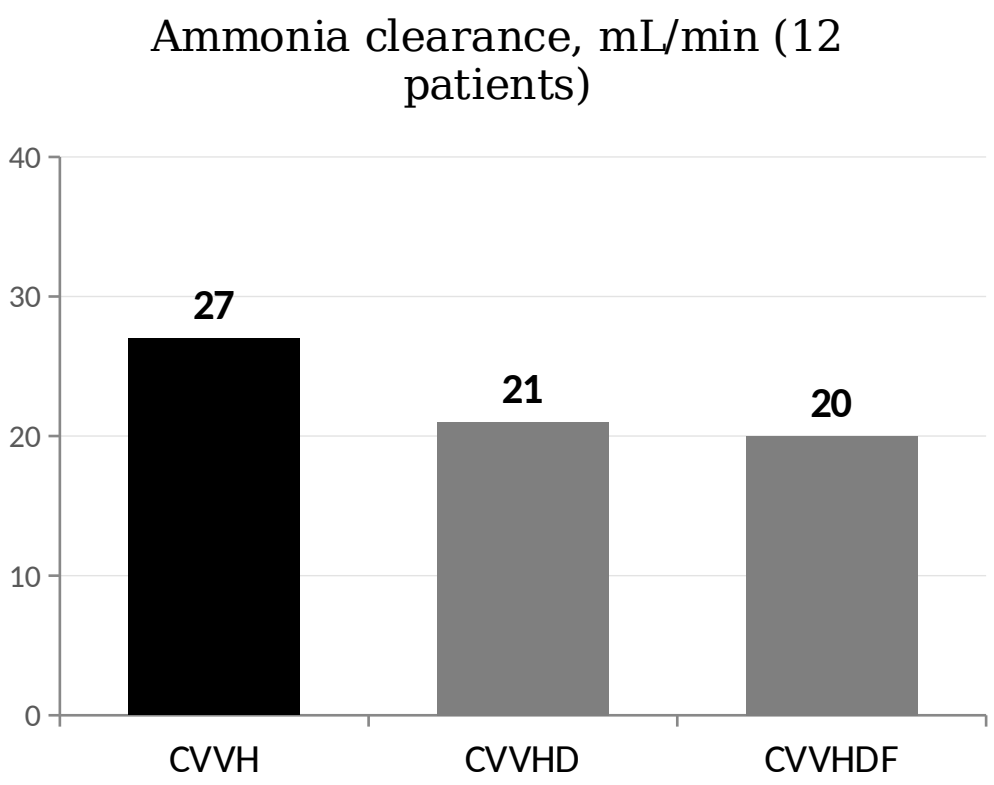
"CVVH doesn't clear ammonia"

True or false?


FALSE

"CVVH doesn't clear ammonia"

- Ammonia is tiny (17 Da), water soluble and not protein bound
- It crosses the filter with the water (convection, CVVH) or down its gradient (diffusion, CVVHD)
- What matters is the effluent dose and hours on a working circuit, more than the mode
- More effluent, more clearance: **90 vs 35 mL/kg/h** (Slack 2014)
- 54 ALF patients on CRRT: median ammonia **151 → 52 µmol/L** by day 5 (Warrillow 2020)



It does. About half the clearance of urea, so dose and filter life matter

Fisher C et al. Blood Purif 2022 [PMID: 35042216]; Warrillow SJ et al. Crit Care Med 2020 [PMID: 31939790]; Slack AJ et al. Liver Int 2014

Does early CRRT actually help?

	US ALF Study Group (2018)	Pilot RCT (2026)
Design	Registry, observational	Open label, ALF with cerebral edema
Comparison	CRRT vs no CRRT	CRRT within 12 h vs delayed
Finding		
Catch		

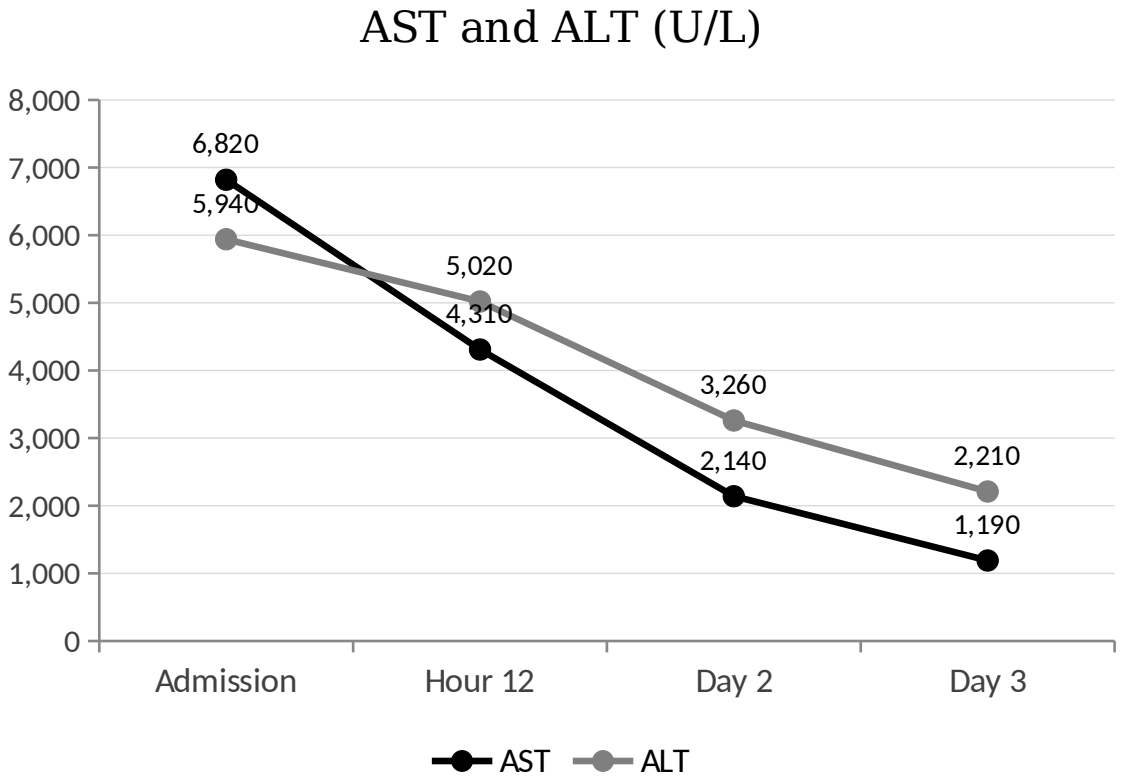
Cardoso FS et al. Hepatology 2018 [PMID: 28859230]; Aliment Pharmacol Ther 2026 [PMID: 42332991]

Does early CRRT actually help?

	US ALF Study Group (2018)	Pilot RCT (2026)
Design	Registry, observational	Open label, ALF with cerebral edema
Comparison	CRRT vs no CRRT	CRRT within 12 h vs delayed
Finding	Lower ammonia, better survival	Same 28-day survival; fewer deaths by day 7
Catch	**Association; selection bias**	**Small, one center, protocol violations**

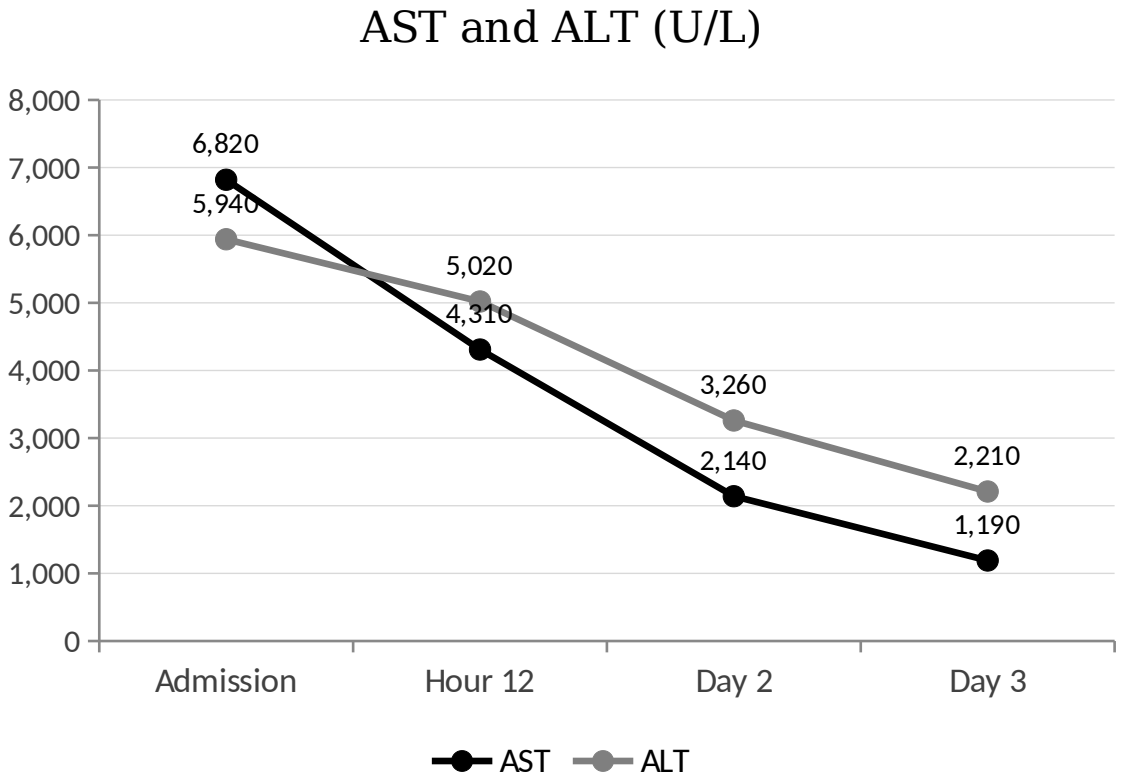
Cardoso FS et al. Hepatology 2018 [PMID: 28859230]; Aliment Pharmacol Ther 2026 [PMID: 42332991]

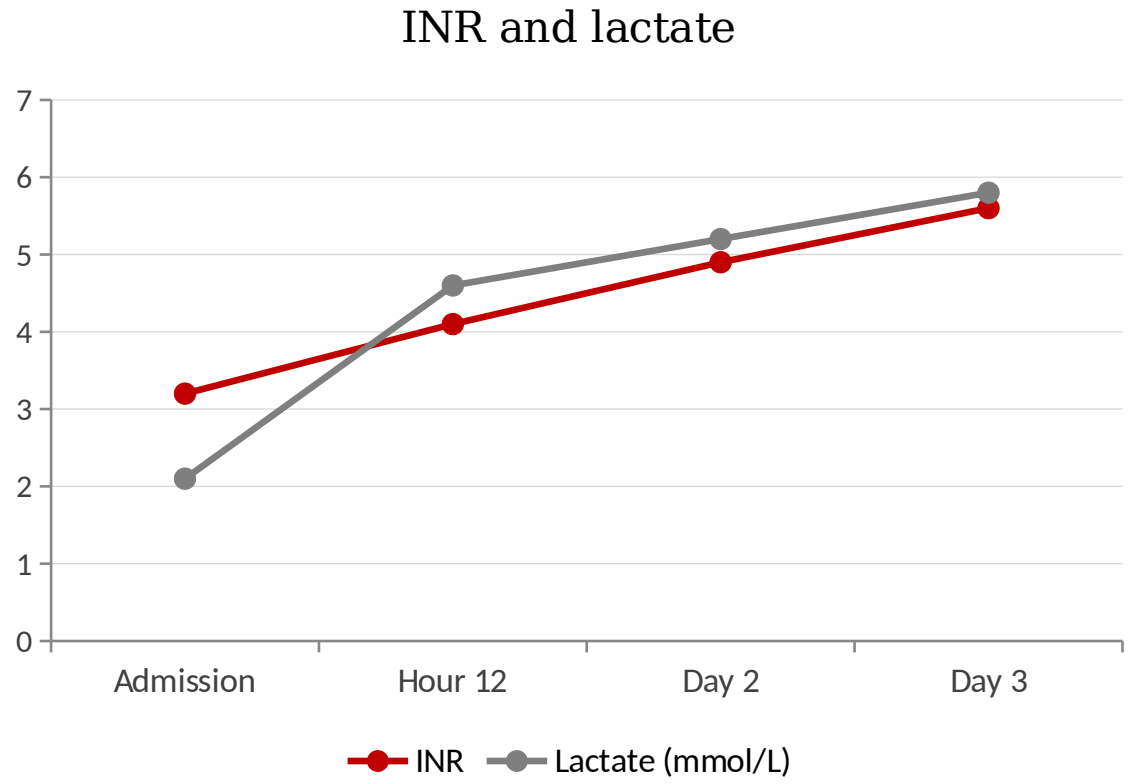
Day 3: the enzymes are falling



Is she getting better?

Day 3: the enzymes are falling





Day 2 arterial pH 7.26 after fluids

No. The INR, the lactate and her brain are all worse

Made-up numbers for teaching

When do you call transplant hepatology?

When she meets King's College criteria?

When MELD is high enough?



When do you call transplant hepatology?

You should have called on admission, when you first suspected ALF

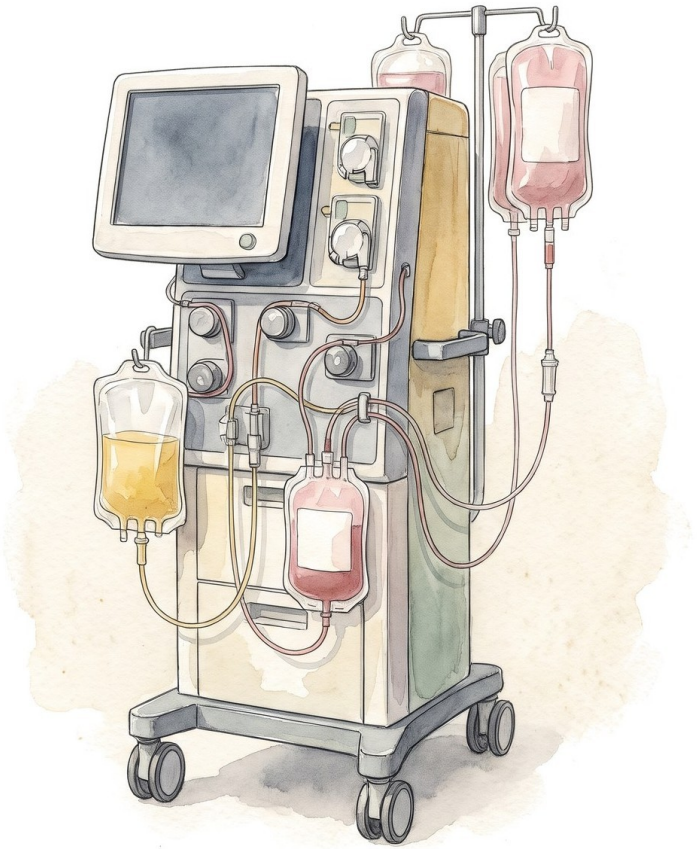
- King's College criteria predict death without transplant. They are specific, not sensitive
- They are not a referral threshold. If you wait for them, the evaluation starts too late
- Patient A: arterial pH **7.26** on day 2 after fluids. She now meets the acetaminophen criteria



O'Grady 1989 criteria, as summarized in EASL 2017 [PMID: 28417882]; full criteria in the appendix

Does plasma exchange help in ALF?

	Larsen 2016	Burke 2025
Design	RCT, 182 ALF	UK, 7 centers, 378 ALF
Plasma exchange	High volume × 3 days	120 patients, matched
Survival	58.7% vs 47.8% (HR 0.56)	51.4% vs 62.6% (p = 0.12)
Catch	**Not blinded; no ACLF**	**Retrospective**



Better blood pressure, no proven survival benefit

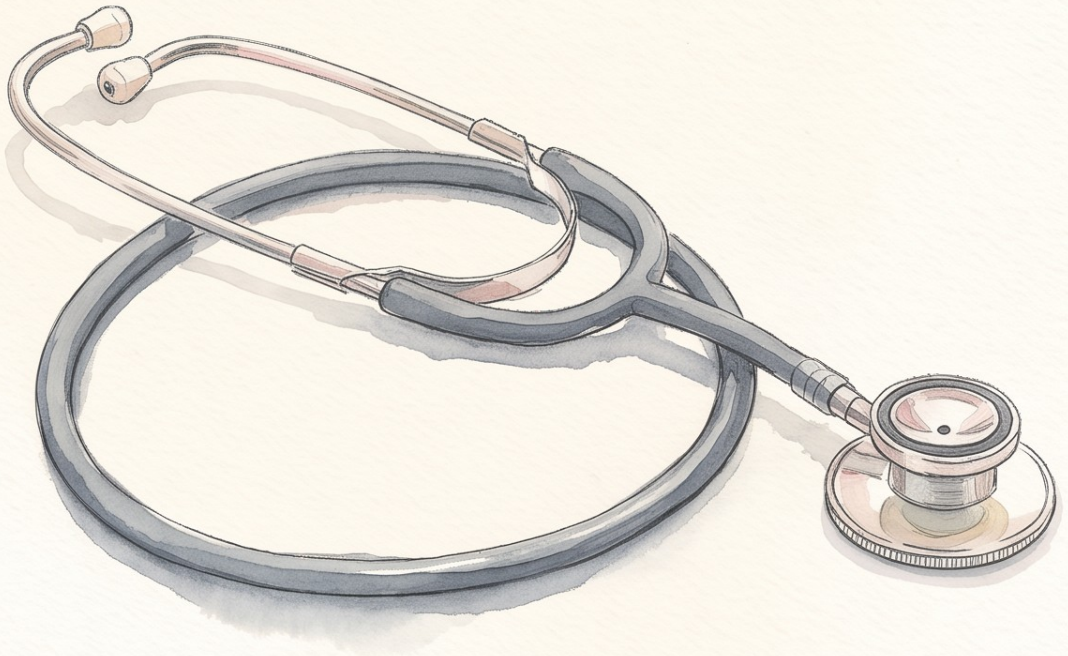
Larsen FS et al. J Hepatol 2016 [PMID: 26325537]; Burke L et al. J Hepatol 2025;82:615-21 [PMID: 39362282]

Bed 4 on day 3

Problem	What she's on	What we watch
Acetaminophen ALF, pH 7.26	NAC; listed 1A on day 2	Time to an offer
HE III-IV, ammonia 182	Intubated, CO2 normal, Na 145-155, CRRT	Pupils, ammonia, Na
Lactate 5.8 and rising	Volume, MAP, look for infection	Lactate, MAP, pressors
INR 5.6, not bleeding	**No plasma**	Bleeding, INR trend
Infection risk	Surveillance cultures	Low bar for antimicrobials

Made-up numbers for teaching

Any questions so far?



Bed 7

56M, cirrhosis and fever



56M, cirrhosis and fever

- Alcohol-related cirrhosis, ascites on diuretics; last drink 3 weeks ago
- Temp 38.4 °C, HR 112, BP 82/46
- RR 26, SpO2 92% on a 35% Venturi mask; PaO2 74
- Confused, asterixis (grade II)
- Urine output 15 mL/h

	Now	Baseline
Bilirubin (mg/dL)	5.8	2.1
INR	2.1	1.6
Creatinine (mg/dL)	2.6	0.9
Sodium (mmol/L)	127	133
WBC (/µL)	16,200	
Platelets (/µL)	62,000	
Lactate (mmol/L)	3.9	

What's your next step?

56M, cirrhosis and fever

- Alcohol-related cirrhosis, ascites on diuretics; last drink 3 weeks ago
- Temp 38.4 °C, HR 112, BP 82/46
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WBC (/µL)	16,200	
Platelets (/µL)	62,000	
Lactate (mmol/L)	3.9	

What's your next step?

Find the source: cultures, antibiotics

What tipped him over?

INR 2.1, platelets 62,000. Do you tap him?



What tipped him over?

- **Yes, now. No plasma, no platelets**
- Ascitic fluid: **PMN 1,240/mm³**. Spontaneous bacterial peritonitis

Common triggers

- Infection: SBP, pneumonia, urine, lines, C. difficile
- Alcohol-associated hepatitis
- GI bleeding
- Drugs: NSAIDs, sedatives, too much diuretic
- HBV flare, ischemia
- No trigger is found in some patients. Repeat the search if he gets worse



EASL ACLF 2023 [PMID: 37364789]; SCCM 2023 [PMID: 37052436]

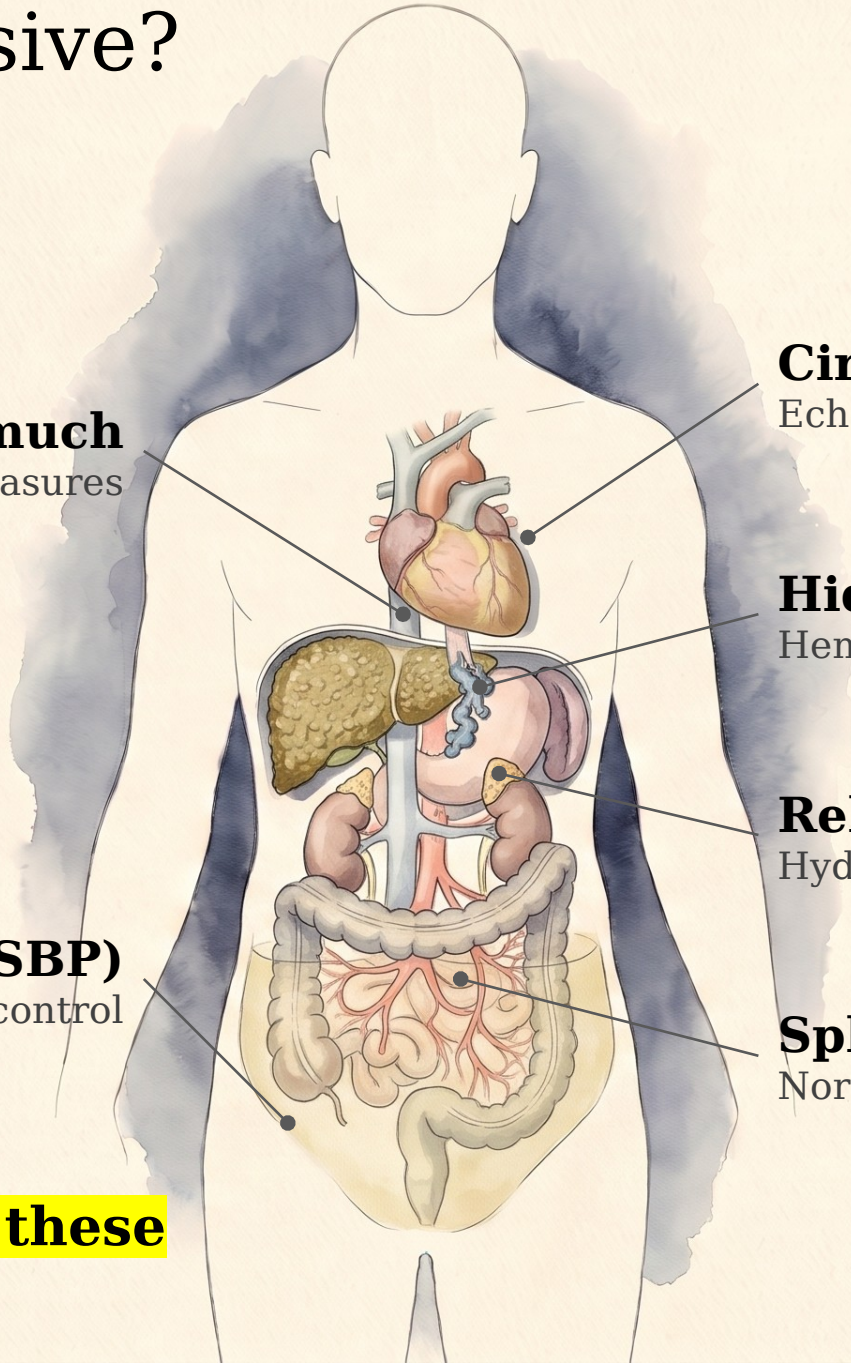
Why is he hypotensive?



Why is he hypotensive?

Too little volume, or too much
Ultrasound and dynamic measures

Sepsis (SBP)
Cultures, antibiotics within 1 h, source control



Cirrhotic cardiomyopathy
Echo

Hidden bleeding
Hemoglobin, rectal exam, think varices

Relative adrenal insufficiency
Hydrocortisone if shock is refractory

Splanchnic vasodilation
Norepinephrine, MAP 65, adjust to perfusion

Usually more than one of these

ALF, decompensation or ACLF?

	Acute liver failure	Acute decompensation	ACLF
Liver before			
Defined by			
Usually kills			
ICU focus			
Transplant			

EASL ALF 2017 [PMID: 28417882]; EASL ACLF 2023 [PMID: 37364789]

ALF, decompensation or ACLF?

	Acute liver failure	Acute decompensation	ACLF
Liver before	No cirrhosis	Cirrhosis	Cirrhosis or chronic liver disease
Defined by	INR ≥ 1.5 + HE	Ascites, bleeding, HE, jaundice	Decompensation + organ failure
Usually kills	Brain, sepsis	Bleeding, infection	Infection, shock, organ failure
ICU focus	Brain, ammonia, early CRRT	Treat the trigger	Source, shock, kidney vs lung
Transplant	Urgent listing	Refer by MELD or complication	Narrow window; reassess daily

EASL ALF 2017 [PMID: 28417882]; EASL ACLF 2023 [PMID: 37364789]

Count the failing organs

Organ	Failure (EASL-CLIF)	Patient B
Liver	Bilirubin ≥ 12 mg/dL	
Kidney	Creatinine ≥ 2.0 mg/dL or on RRT	
Brain	Encephalopathy grade III-IV	
Coagulation	INR ≥ 2.5	
Circulation	Vasopressors (or terlipressin)	
Lungs	P/F ≤ 200, S/F ≤ 214, or ventilated (not for HE)	

CLIF-C organ failure score, EASL ACLF 2023 [PMID: 37364789]; CANONIC [PMID: 23474284]

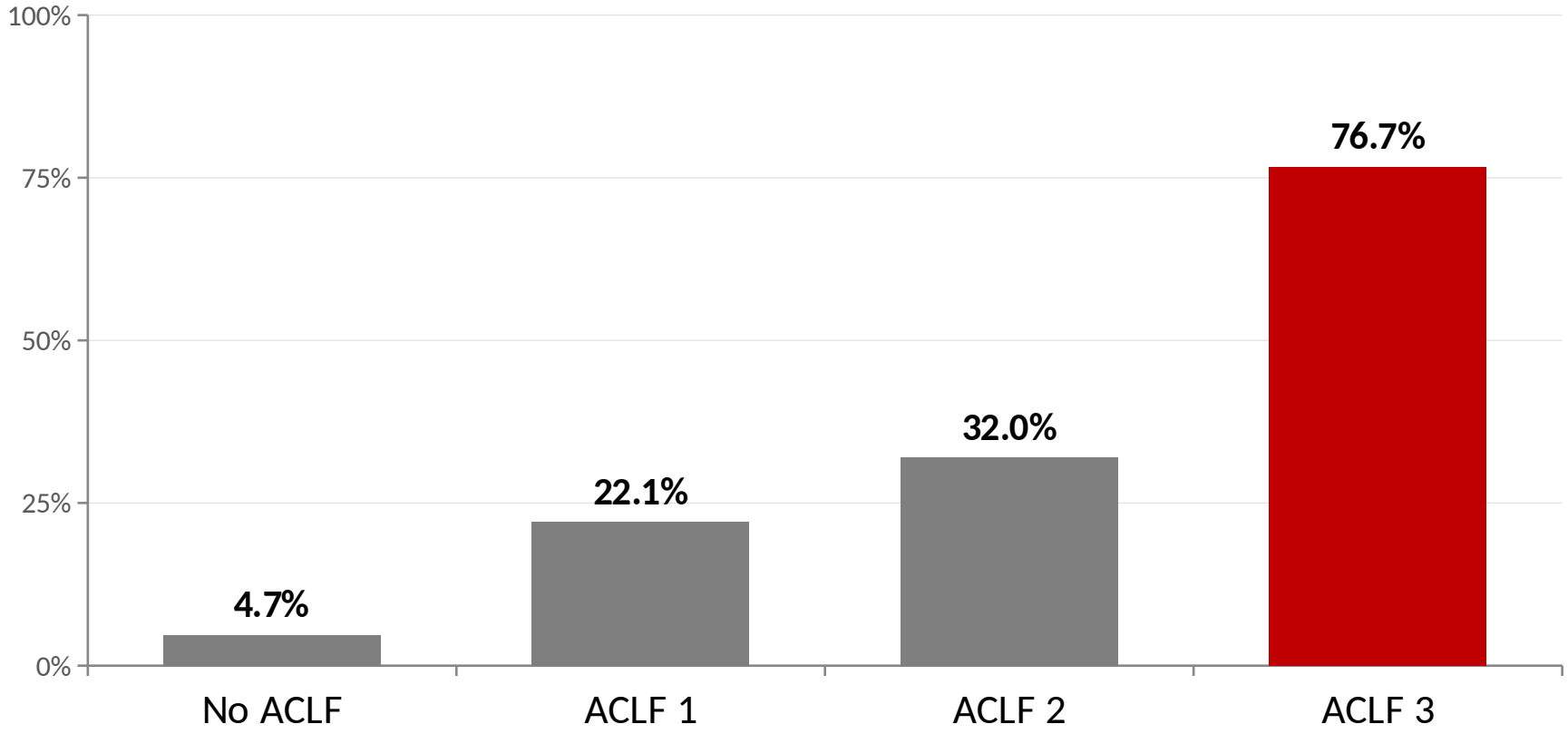
Count the failing organs

Organ	Failure (EASL-CLIF)	Patient B
Liver	Bilirubin ≥ 12 mg/dL	5.8: no
Kidney	Creatinine ≥ 2.0 mg/dL or on RRT	**2.6: yes**
Brain	Encephalopathy grade III-IV	Grade II: no
Coagulation	INR ≥ 2.5	2.1: no
Circulation	Vasopressors (or terlipressin)	**Norepinephrine: yes**
Lungs	P/F ≤ 200, S/F ≤ 214, or ventilated (not for HE)	P/F 211: no

Two organ failures: ACLF grade 2

CLIF-C organ failure score, EASL ACLF 2023 [PMID: 37364789]; CANONIC [PMID: 23474284]

28-day mortality by ACLF grade



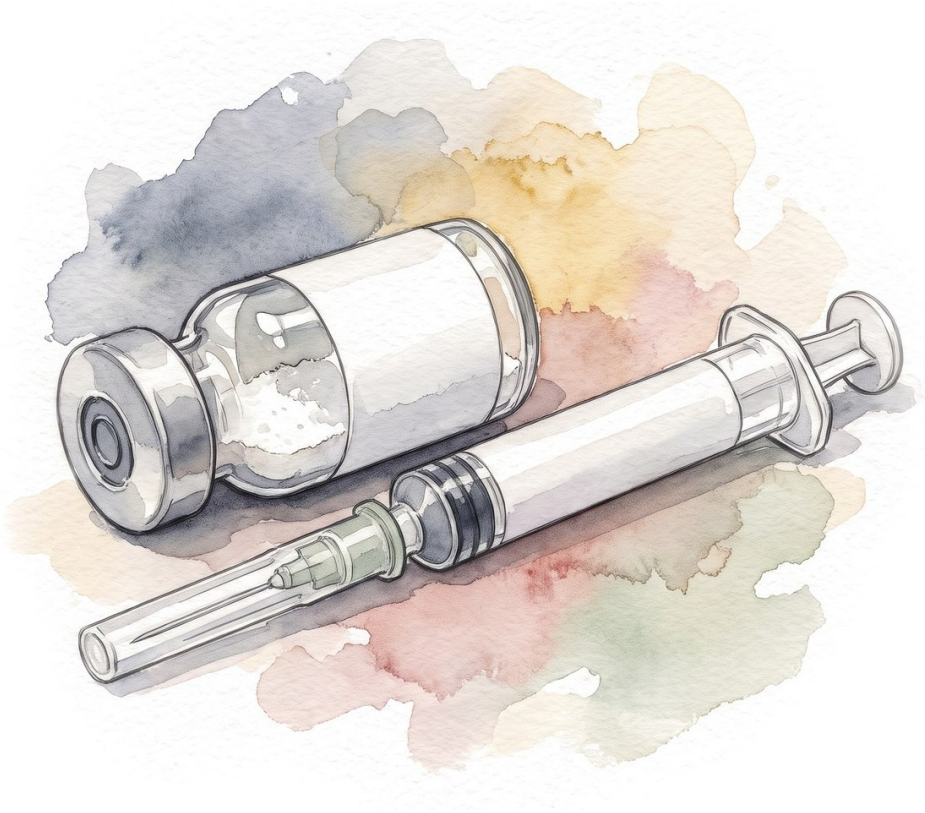
CANONIC, 1,343 patients, 2013

Moreau R et al. Gastroenterology 2013 [PMID: 23474284]

Does he have HRS?

12 hours later: creatinine 3.0, urine 10 mL/h, still on norepinephrine

HRS? Start terlipressin?



Does he have HRS?

12 hours later: creatinine 3.0, urine 10 mL/h, still on norepinephrine

HRS-AKI (ADQI-ICA 2024)	**Bed 7, day 1**	
Cirrhosis with ascites		
AKI by creatinine or urine output		
No response to 24 h of volume, if needed		
No other main cause (shock, toxins, structural)		

Go down the list for him. Which boxes does he tick?

ADQI-ICA consensus, J Hepatol 2024 [PMID: 38527522]

Does he have HRS?

12 hours later: creatinine 3.0, urine 10 mL/h, still on norepinephrine

HRS-AKI (ADQI-ICA 2024)	**Bed 7, day 1**	
Cirrhosis with ascites	Yes	✅
AKI by creatinine or urine output	Cr 0.9 → 3.0, urine 10 mL/h	✅
No response to 24 h of volume, if needed	2 L on arrival; too early	❌
No other main cause (shock, toxins, structural)	**Septic shock on norepinephrine**	❌

Not HRS yet. Treat the shock first

ADQI-ICA consensus, J Hepatol 2024 [PMID: 38527522]

Day 2: worse kidney, worse lungs

	Admission	Day 2
Albumin received		110 g (SBP dose, 1.5 g/kg at 73 kg) + 2 L crystalloid
Breathing	SpO2 92%, Venturi 35%	**SpO2 86% on Venturi 50%, RR 32, crackles**
Ultrasound		Diffuse B-lines, IVC full
Creatinine (mg/dL)	2.6	3.1
Urine output	15 mL/h	10 mL/h
Norepinephrine (mcg/kg/min)	0.15	0.22

More albumin? Terlipressin? Something else?

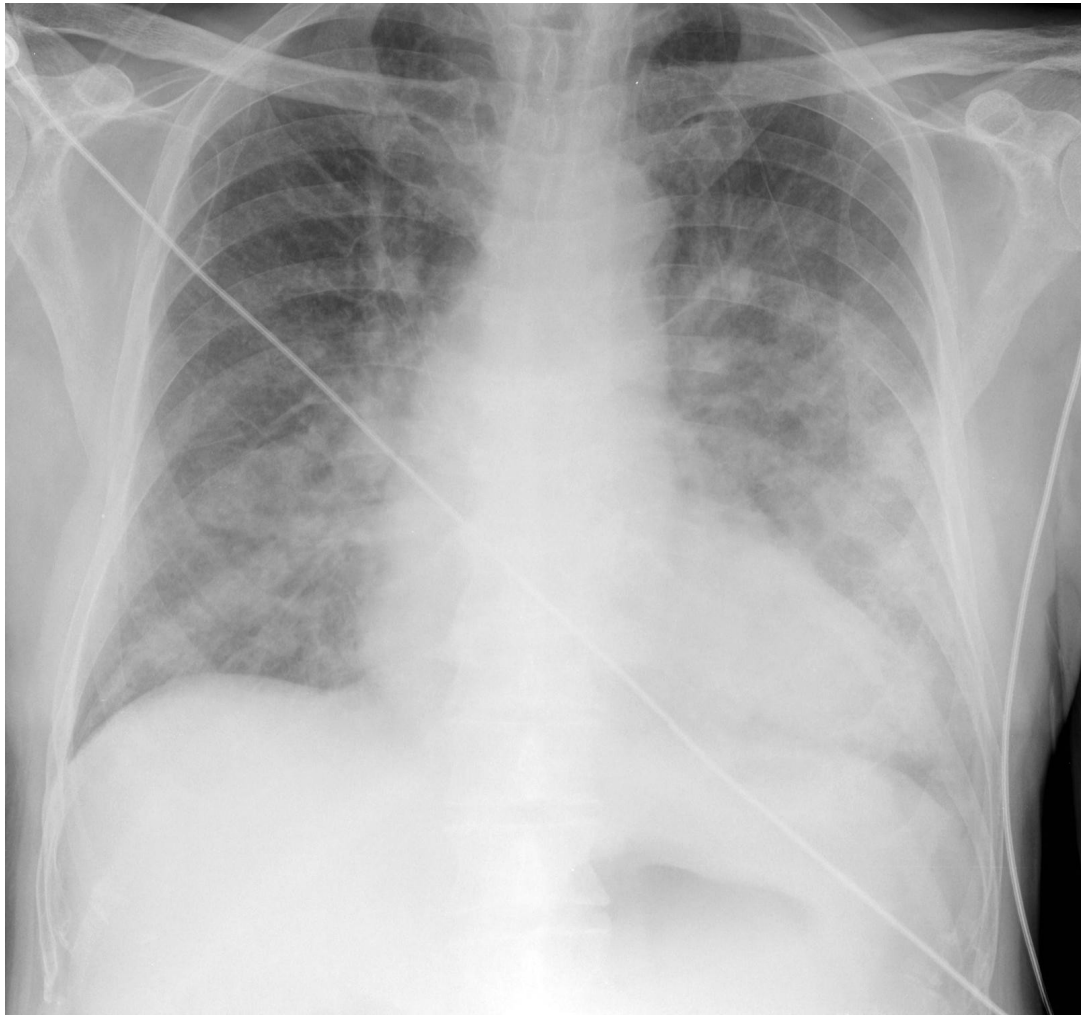
Day 2: worse kidney, worse lungs

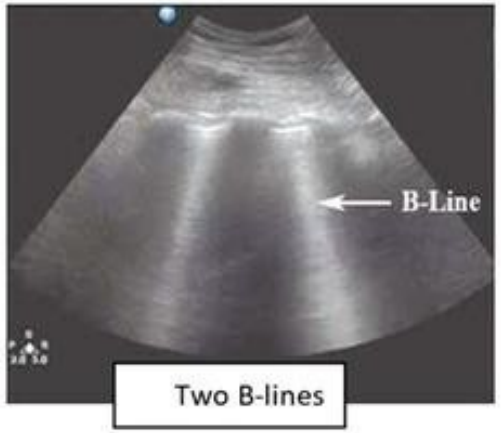
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Creatinine (mg/dL)	2.6	3.1
Urine output	15 mL/h	10 mL/h
Norepinephrine (mcg/kg/min)	0.15	0.22

First, look at his oxygen

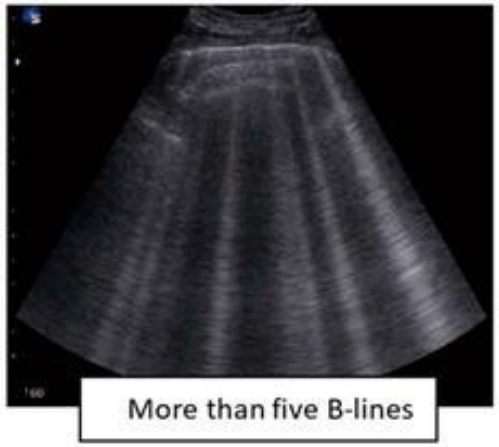
Now on high-flow, FiO2 0.6, SpO2 90%: SpO2/FiO2 150. That's a third organ failure

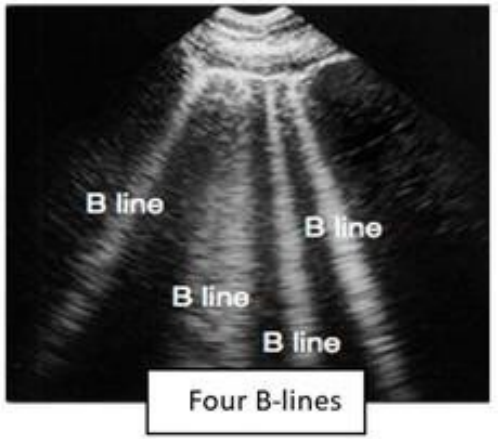
Day 2: what do his lungs look like?







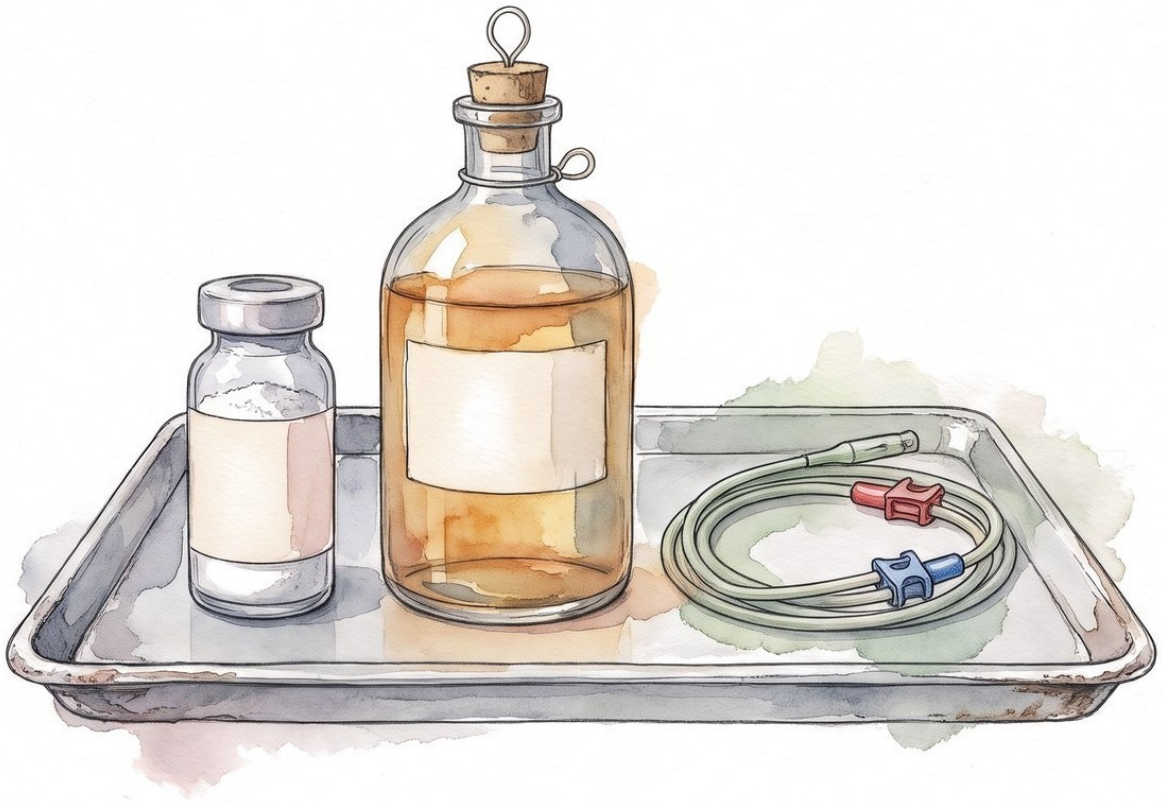




Images for teaching, not this patient

What would you do for his kidney today?

Terlipressin, more albumin, or dialysis?



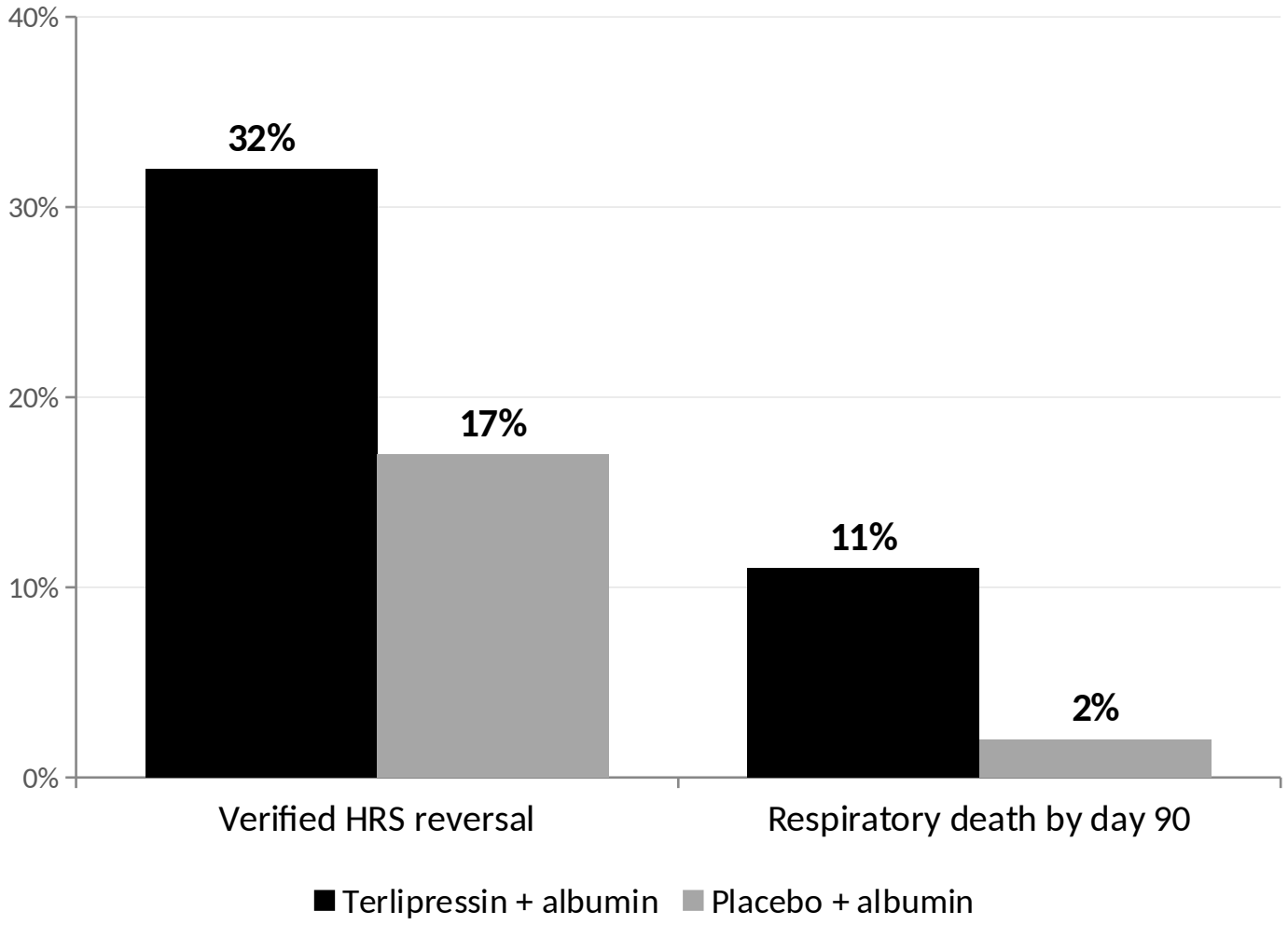
What would you do for his kidney today?

- **Terlipressin: not for him today**
  - Label: don't start if SpO2 < 90% or breathing is getting worse
  - Label: avoid in ACLF grade 3 and in volume overload
- Stop the albumin while he is wet
- He is already on a vasoconstrictor. Norepinephrine, titrated to perfusion
- RRT if he stays overloaded. For HRS, RRT is a bridge to transplant: if he isn't a candidate, talk about goals before you start

Check oxygen and volume before any kidney treatment

TERLIVAZ prescribing information, revised 6/2026; ADQI-ICA 2024 [PMID: 38527522]

Does HRS reversal mean survival?



CONFIRM: type 1 HRS, 300 patients

- More kidneys recovered
- **More patients died of respiratory failure**
- All-cause 90-day mortality was not different
- Respiratory failure as an adverse event: about 14% vs 5% (label)

Wong F et al. NEJM 2021 [PMID: 33657294]; TERLIVAZ prescribing information, revised 6/2026

Should we treat the albumin number?

Study	777 inpatients: albumin to ≥ 30 g/L vs standard care
Finding	Infection, kidney dysfunction or death: 29.7% vs 30.2%
The catch	**More serious adverse events, including pulmonary edema**

When albumin is indicated

- SBP: 1.5 g/kg on day 1, 1 g/kg on day 3
- Paracentesis over 5 L: 6 to 8 g per liter removed
- HRS-AKI: with a vasoconstrictor, dosed to volume status

China L et al. NEJM 2021 [PMID: 33657293]; ADQI-ICA 2024 [PMID: 38527522]

What changed since yesterday?

	Day 1	Day 2	Day 3	Day 5
Norepinephrine (mcg/kg/min)	0.15	0.22	0.12	Off
Oxygen, SpO2	Venturi 0.35, 92%	HFNC 0.6, 90%	HFNC 0.4, 95%	2 L, 97%
Lung failure?	No (P/F 211)	Yes (S/F 150)	No (S/F 238)	No
Kidney	Cr 2.6	Cr 3.1	CRRT for fluid	On CRRT
Encephalopathy	Grade II	Grade II	Grade I	Grade I
Bilirubin / INR	5.8 / 2.1	6.4 / 2.3	6.1 / 2.2	5.2 / 2.0
ACLF grade	2	**3**	2	?

Better or worse? What do you tell his family?

Made-up numbers for teaching. CLIF-C ACLF score [PMID: 24950482]; EASL ACLF 2023 [PMID: 37364789]

What changed since yesterday?

	Day 1	Day 2	Day 3	Day 5
Norepinephrine (mcg/kg/min)	0.15	0.22	0.12	Off
Oxygen, SpO2	Venturi 0.35, 92%	HFNC 0.6, 90%	HFNC 0.4, 95%	2 L, 97%
Lung failure?	No (P/F 211)	Yes (S/F 150)	No (S/F 238)	No
Kidney	Cr 2.6	Cr 3.1	CRRT for fluid	On CRRT
Encephalopathy	Grade II	Grade II	Grade I	Grade I
Bilirubin / INR	5.8 / 2.1	6.4 / 2.3	6.1 / 2.2	5.2 / 2.0
ACLF grade	2	**3**	2	**1**

- The grade at days 3 to 7 predicts outcome better than the grade at admission
- Persistent ACLF 3 at days 3 to 7, especially with four or more organ failures, has a very high mortality without transplant
- If he is not a transplant candidate, discuss goals of care with the family and involve palliative care

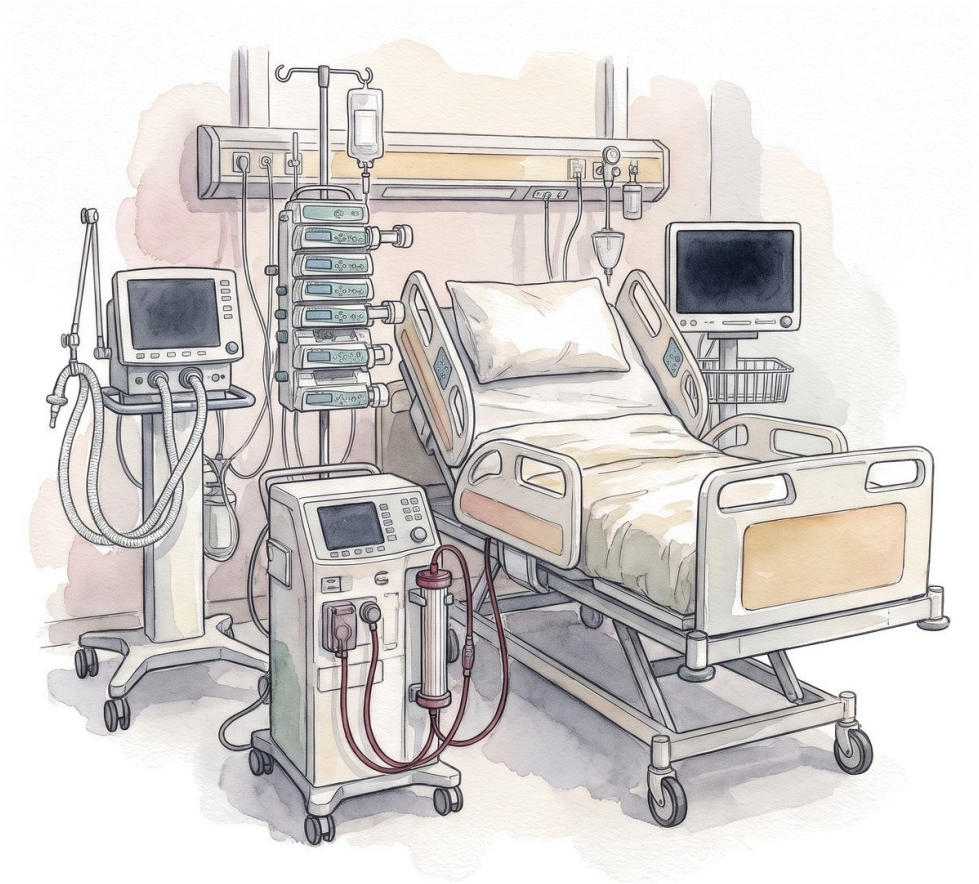
Made-up numbers for teaching. CLIF-C ACLF score [PMID: 24950482]; EASL ACLF 2023 [PMID: 37364789]

Transplant in ACLF



Is he too sick for a transplant?

ACLF 3 on a ventilator. Would you call?



Is he too sick for a transplant?

- A ventilator, pressors or dialysis do not by themselves exclude transplant
- Selected patients with ACLF grade 3 do well after transplant, if it happens early

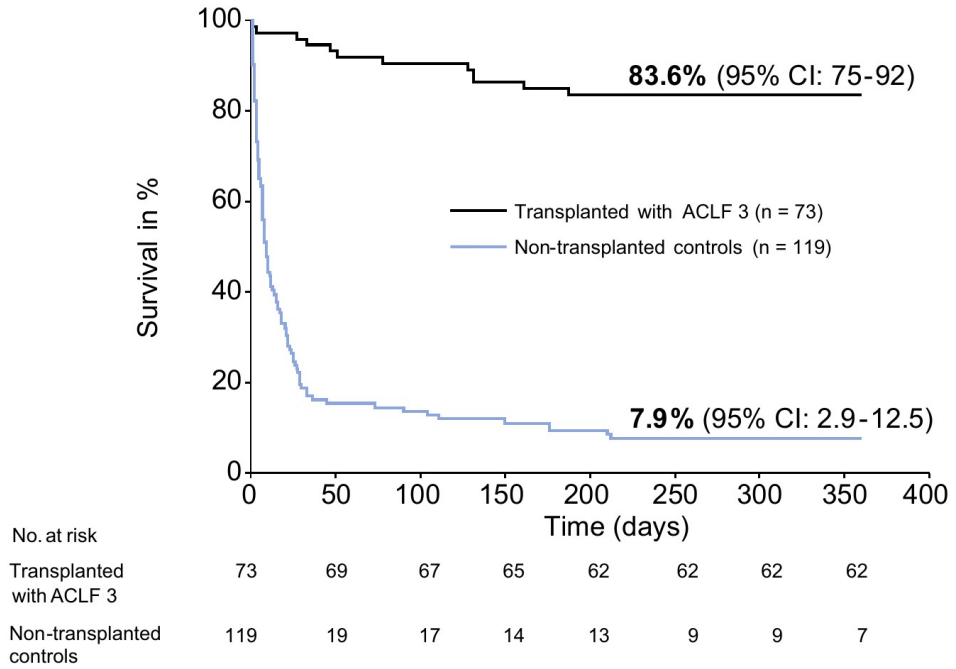
What makes transplant unlikely

- Uncontrolled infection
- Rising vasopressor needs, severe respiratory failure
- Irreversible brain injury
- Frailty and comorbidity
- Alcohol: centers now evaluate individually rather than by a fixed abstinence rule

Research Article

EASL JOURNAL OF HEPATOLOGY

Liver transplantation in the most severely ill cirrhotic patients: A multicenter study in acute-on-chronic liver failure grade 3



Selected patients, not randomized

Artru F et al. J Hepatol 2017 [PMID: 28645736]; Lancet Gastroenterol Hepatol 2024 [PMID: 38309288]; AASLD/AST 2026 [PMID: 41405234]

What happened to them

Bed 4



Day 0
Admitted, NAC

Day 1
Intubated, CRRT

Day 2
Listed 1A

Day 4
Transplanted

Day 19
Home

Bed 7



Day 1
SBP, ACLF 2

Day 2
ACLF 3

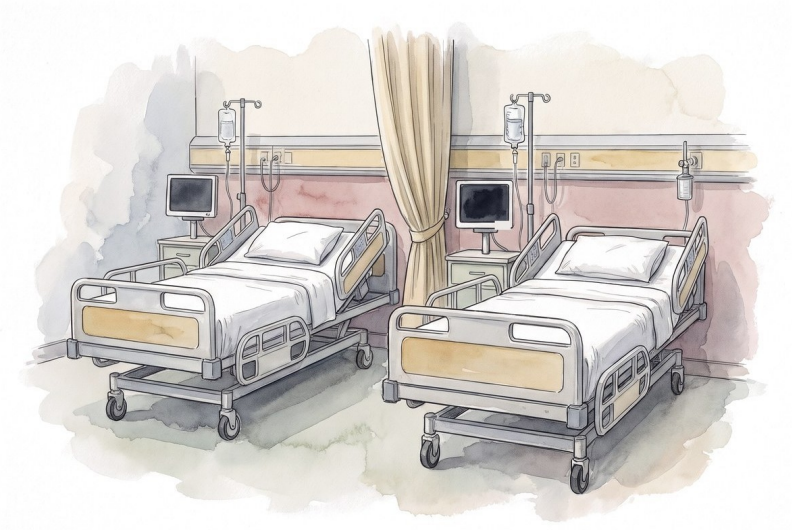
Day 5
Off pressors, ACLF 1

Day 12
Off CRRT

Day 21
Home, transplant clinic

Made-up outcomes for teaching

Back to our patients

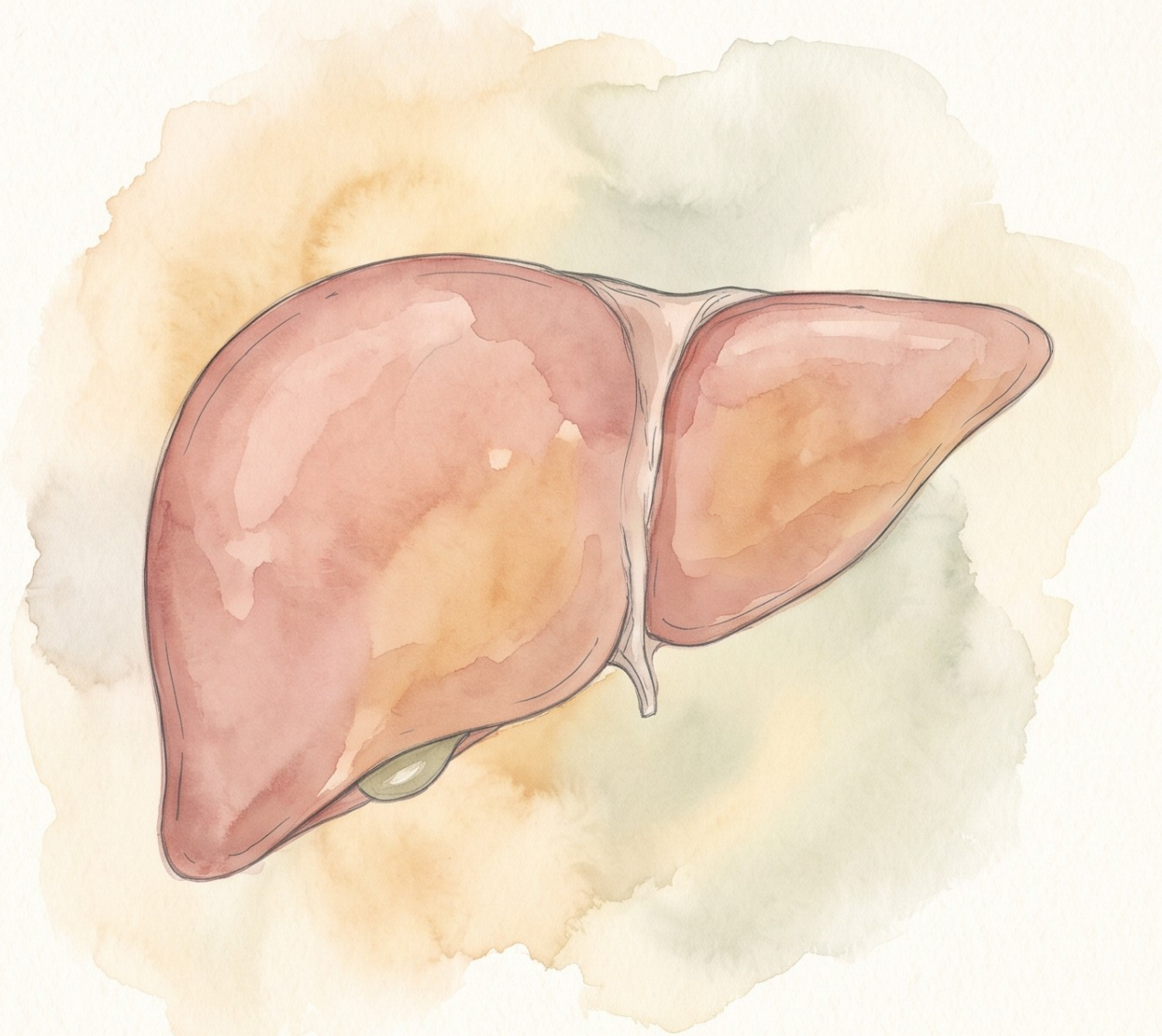


- Who needed transplant hepatology before the labs?
- Why did a falling ALT worry us more?
- When is CRRT right before the creatinine rises?
- Why doesn't cirrhosis lactulose data apply to ALF?
- Why not albumin and terlipressin for bed 7?
- What would make you change yesterday's plan?

Summary

- High AST and ALT alone are not liver failure. Get an INR and examine the patient
- Stabilize, treat the cause and call transplant hepatology in the same hour
- ALF: **protect the brain**. Intubate at grade III, keep the CO2 normal, follow your center's sodium plan, and start CRRT for rising ammonia
- ACLF: count the organs, find the trigger
- ACLF: check oxygenation and volume status before albumin or terlipressin
- Reassess organ failures daily. The trend matters more than the admission score





Questions?

Slides (PDF)

Appendix: West Haven grades

Grade	What you see
I	Short attention, mild confusion, reversed sleep, subtle personality change
II	Lethargy, disorientation to time, obvious personality change, asterixis
III	Somnolent but rousable, grossly disoriented, bizarre behavior
IV	Coma

Appendix: King's College criteria

Acetaminophen	Not acetaminophen
Arterial pH < 7.30 after fluids *or all three within 24 hours:* INR > 6.5 Creatinine > 3.4 mg/dL Grade III-IV encephalopathy *(Modified: lactate > 3.5 at 4 h or > 3.0 at 12 h after fluids)*	**INR > 6.5** *or any three of:* Age < 10 or > 40 Drug reaction or unknown cause Jaundice to encephalopathy > 7 days INR > 3.5 Bilirubin > 17.5 mg/dL

Original O'Grady criteria; the lactate line is the later modification

O'Grady 1989 criteria, as summarized in EASL 2017 [PMID: 28417882] and update 2022 [PMID: 36001996]

Appendix: arterial ammonia studies in ALF

Study (arterial, µmol/L)	Patients	What they found	The catch
Clemmesen 1999	44 ALF	**Herniated: mean 230 vs 118; all above 146**	Small, one center
Bhatia 2006	80 ALF	≥ 124 on admission predicted death	Endpoint was death, not ICH
Bernal 2007	165, HE III-IV	> 100: severe HE. **> 200: ICH in 55%**	200 missed most ICH
Kumar 2012	295 ALF	**≥ 122 for 3 days: survival 23% vs 72%**	Trend beat a single value

Above 150 to 200 and not falling: worry about the brain

ICH can occur below 200. Interpret the level with the HE grade and the trend

Clemmesen 1999 [PMID: 10051463]; Bhatia 2006 [PMID: 16024550]; Bernal 2007 [PMID: 17685471]; Kumar 2012 [PMID: 22521861]

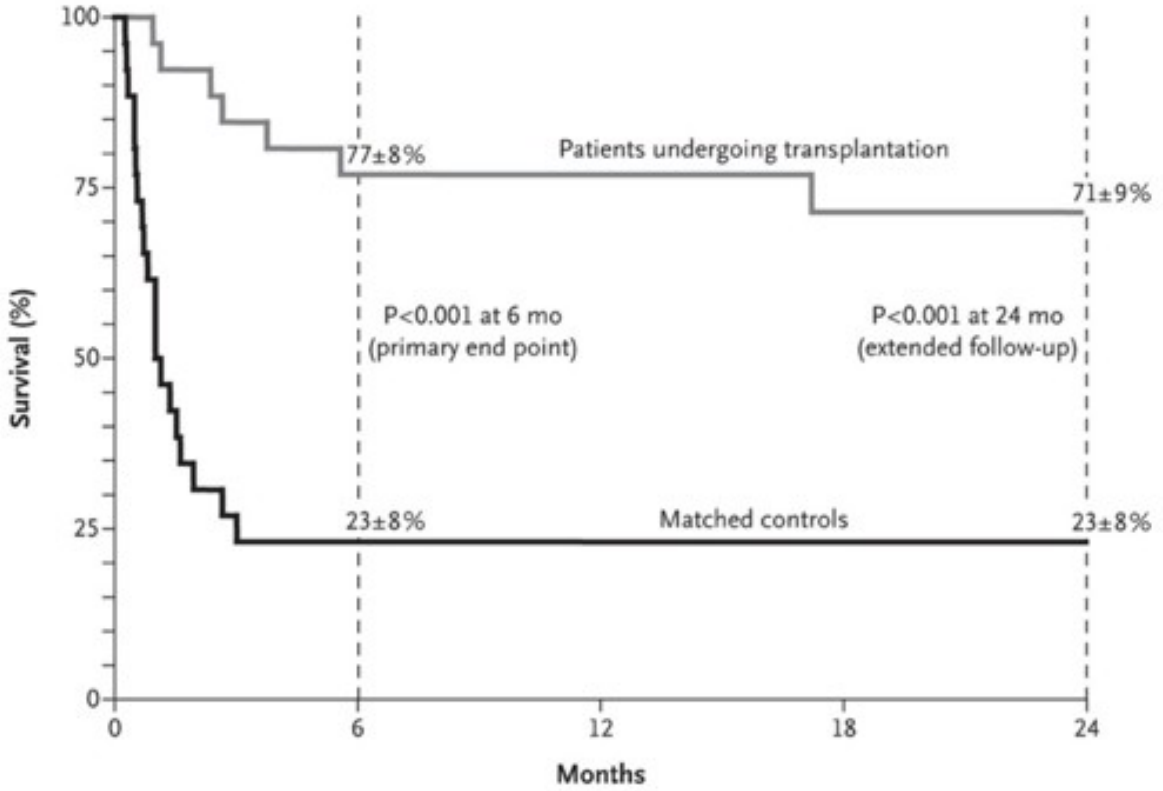
Appendix: which ACLF?

	EASL-CLIF	NACSELD	APASL
Who	Decompensated cirrhosis	Cirrhosis, inpatient	CLD ± cirrhosis
Defined by	Organ failures (CLIF-C)	≥ 2 extrahepatic failures	Liver insult: jaundice + INR ≥ 1.5
Picks up	Broader, earlier	Sicker patients	A primary liver insult

They find different patients. Say which one you're using

Kyoto consensus 2025 [PMID: 39961976]; ACG ACLF 2022 [PMID: 35006099]; AASLD 2024 [PMID: 37939273]

Appendix: early transplant for alcohol-associated hepatitis



No. at Risk					
Patients undergoing transplantation	26	20	15	14	13
Matched controls	26	6	6	5	4

- 26 patients with a first episode of severe alcohol-associated hepatitis, not responding to steroids
- Highly selected: first liver-related event, close family support, no severe other illness, agreed to abstinence
- 6-month survival **77% vs 23%**
- Recent drinking alone is not a reason to skip the call

Mathurin P et al. N Engl J Med 2011 [PMID: 22070476]

Appendix: calling transplant hepatology

Call first. Gather this while you wait for the call back

Patient A (ALF)	Patient B (ACLF)
Cause, days from jaundice to HE	Cause of cirrhosis, recent alcohol use
Encephalopathy grade, pupils, sedation	Precipitant and source control
Airway and ventilator settings	Organ failures today and yesterday
Ammonia, sodium, CRRT	Pressor dose, oxygen, RRT
INR, lactate, pH, glucose trends	MELD, CLIF-C scores
Pressors, infection	Frailty, comorbidities
Social support, intent of ingestion	Prior transplant evaluation, support

Appendix: sources (1)

- SCCM ALF/ACLF ICU guidelines, part 1. Crit Care Med 2020 [PMID: 32058387]
- SCCM ALF/ACLF ICU guidelines, part 2. Crit Care Med 2023 [PMID: 37052436]
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- EASL hepatic encephalopathy guideline. J Hepatol 2022 [PMID: 35724930]
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Appendix: sources (2)

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- Lee WM et al. NAC in non-acetaminophen ALF. Gastroenterology 2009 [PMID: 19524577]
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- Burke L et al. Plasma exchange in a real-world ALF cohort. J Hepatol 2025;82:615-21 [PMID: 39362282]
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- Artru F et al. Transplant in ACLF grade 3. J Hepatol 2017 [PMID: 28645736]
- Mathurin P et al. Early transplant for alcoholic hepatitis. NEJM 2011 [PMID: 22070476]
- China L et al. ATTIRE. NEJM 2021 [PMID: 33657293]
- Liver transplantation for ACLF. Lancet Gastroenterol Hepatol 2024 [PMID: 38309288]
- AASLD/AST candidate evaluation guideline. Hepatology 2026 [PMID: 41405234]
- Kyoto consensus on ACLF. Hepatol Int 2025 [PMID: 39961976]
- TERLIVAZ (terlipressin) prescribing information, revised 6/2026